Multiculturalism, Social Justice, Advocacy, and Values

3

LEARNING OBJECTIVES

3.1 Describe counselors' responsibilities for promoting multicultural competence, social justice, and advocacy.

3.2 Explain why dealing with value conflicts has become a central issue in counseling.

FOCUS QUESTIONS

1. What do you see as the characteristics of a culturally competent counselor?
2. In what ways is multicultural competence an ethical issue?
3. How would you define *social justice*?
4. How can you ensure that you are competent to provide counseling services to the culturally diverse clients who may seek your assistance?
5. Do you believe a counselor should be allowed to refuse to provide counseling services to a client whose beliefs or behaviors conflict with the personal value system of the counselor?

A core professional value of counselors, as articulated in the Preamble to the ACA *Code of Ethics* (2014), is "honoring diversity and embracing a multicultural approach" in all aspects of our work. If you are a counseling student or a new professional, you may not be aware that this foundational commitment to diversity has not always been a hallmark of our profession. Over the past 7 decades, the counseling profession has evolved significantly in its stance toward multiculturalism and social justice. The profession has moved from a position of cultural encapsulation to an awareness of our ethical responsibility to serve diverse clients effectively by practicing in a multiculturally competent manner and to an emphasis on social justice and advocacy aimed at improving the lives of all people in our society and across the world (Clark et al., 2022).

In the early years of the development of the counseling profession, the impact of cultural diversity was virtually ignored. Wrenn (1962) was one of the first counseling professionals to draw attention to this issue when he introduced the term *culturally encapsulated counselor*. As Wrenn explained, culturally encapsulated counselors define reality according to one set of cultural assumptions and thus fail to evaluate other viewpoints, becoming locked into stereotypical thinking in the process. During the 1960s, the Civil Rights and women's movements increased societal awareness of equity issues, and counselors began to recognize that cultural issues that had been ignored in society had also been ignored in counseling.

The 1970s saw a growing concern about mental health practitioners' lack of preparedness to counsel diverse clients. The American Counseling Association (ACA) and its

divisions provided leadership in the effort to make mental health services responsive to the needs of a diverse clientele. Throughout the 1980s, multicultural counseling was increasingly recognized as a major discipline within the profession. The need became apparent for counselors to be multiculturally competent practitioners who are able to establish a working alliance that takes into consideration both the personal and the cultural dynamics of the client and the counselor (Lee, 2013). As the twentieth century entered its last decade, the Association for Counselor Education and Supervision (ACES) and the Association for Multicultural Counseling and Development (AMCD), in a joint effort, developed and operationalized the Multicultural Counseling Competencies and Standards (Arredondo et al., 1996). Later, Arredondo and Aricniega (2001) added grounding principles that placed these competencies in an ecological frame. In 2015, AMCD developed the Multicultural and Social Justice Competencies (AMCD, 2015), which are endorsed by ACA.

The twenty-first century has been marked so far by a shift beyond awareness of societal inequities to an emphasis on social justice and advocacy. Multiculturalism, social justice, and advocacy are complementary but distinct concepts that work together interdependently (Fickling & Gonzalez, 2016). Social justice has been defined as a commitment to promoting access and equity that has the aim of empowering and ensuring full participation of all people in a society, especially those who have been marginalized and systematically excluded on the basis of race, ethnicity, gender, age, ability status, sexual or affectional orientation, socioeconomic status, immigration status, national origin, or other characteristic of group membership. Advocacy translates that commitment into action. When issues of power, privilege, and discrimination arise, counselors are called on to address client issues at successively broader levels (Durodoye, 2013). Counselors as advocates are able to intervene to improve the social context that affects clients' lives as well as the lives of individual clients (Feldwisch & Whiston, 2015; Schuster et al., 2021). The ACA Advocacy Competencies, first published in 2003, were updated in 2018. We recommend that you familiarize yourself with both the Multicultural and Social Justice Competencies (2015) and the Advocacy Competencies (2018), which can be found on the ACA website [www.counseling.org].

MULTICULTURAL COMPETENCE, SOCIAL JUSTICE, AND ADVOCACY

Because the counseling profession was slow, historically, to recognize the connection between multicultural competence and ethical behavior, multicultural considerations have been infused only gradually into our profession's code of ethics. Watson et al. (2006) traced this gradual infusion into successive revisions of the ACA *Code of Ethics* over the years. When the organization (then the American Personnel and Guidance Association; APGA) adopted its first code of ethics in 1961, counselors were essentially a monocultural group. No mention was made of cultural awareness as a component of ethical practice in the first version of the *Code*, and successive revisions gave only slight attention to cultural diversity.

As the 1980s drew to a close, it became apparent that the ACA *Code of Ethics* was continuing to overlook the needs of disenfranchised groups. In 1995, the *Code* was revised extensively, and multicultural considerations were infused so that they were prominent. In 2002, David Kaplan, then president of ACA, appointed an Ethics Revision Task Force to propose revisions to the ethics code, with special focus on multiculturalism, diversity, and social justice. The 2005 version of the ACA *Code of Ethics* (ACA, 2005) gave centrality to multiculturalism by addressing diversity in the Preamble and in every major section of the *Code*.

The strong commitment to diversity and multicultural competence that was visible in the 2005 *Code* is evident throughout the current ACA *Code of Ethics* (ACA, 2014). The Preamble articulates the core professional values of counselors, which include "honoring diversity and

embracing a multicultural approach in support of the worth, dignity, potential, and uniqueness of people within their social and cultural contexts" and "promoting social justice." In the Introduction to Section A, which focuses on the counseling relationship, counselors are urged to "actively attempt to understand the diverse cultural backgrounds of the clients they serve. Counselors also explore their own cultural identities and how these affect their values and beliefs about the counseling process." The *Code* clarifies that "multicultural counseling competency is required across all specialties" and that counselors must "gain knowledge, personal awareness, sensitivity, dispositions, and skills pertinent to being a culturally competent counselor in working with a diverse client population" (§ C.2.a.). The *Code* provides additional guidance regarding nondiscrimination in several standards, including § C.5., which states that counselors do not condone or engage in discrimination "based on age, culture, disability, ethnicity, race, religion/spirituality, gender, gender identity, sexual orientation, marital status/partnership, language preference, socioeconomic status or any basis proscribed by law." Lee (2015) identified 26 standards in the 2014 *Code* that address diversity issues, including specific standards related to confidentiality, assessment, diagnosis, relationship boundaries, technology, supervision, counselor training, and research. The concept of social justice, which did not specifically appear in the ACA *Code of Ethics* until the 2014 revision (Francis & Farrell, 2022), is articulated in the definition of social justice as "the promotion of equity for all people and groups for the purpose of ending oppression and injustice affecting clients, students, counselors, families, communities, schools, workplaces, governments, and other social and institutional systems."

The 2014 *Code of Ethics* also addresses the commitment of counselors to advocate on behalf of their clients. Advocacy involves confronting the social issues that negatively affect clients and working to eradicate systems that perpetuate discrimination. Standard A.7.a. addresses this obligation in the statement that "when appropriate, counselors advocate at individual, group, institutional, and societal levels to address potential barriers and obstacles that inhibit access and/or the growth and development of clients." As noted previously, to assist counselors in carrying out advocacy activities ethically and effectively, a set of Advocacy Competencies, updated in 2018, is available on the ACA website (www.counseling.org). Further explanation of competency for advocacy at all levels has been given by Lewis et al. (2011) and in a special issue of the *Journal of Counseling and Development* (2020).

The Impact of the COVID-19 Pandemic on Multiculturalism, Social Justice, Advocacy, and Values

It is abundantly clear that the COVID-19 pandemic took a disproportionate toll on marginalized populations in our society (Litam & Hipolito-Delgado, 2021; Lund, 2020; Melamed et al., 2020; Perkins et al., 2021; Saia et al., 2021). Members of Black, Indigenous, and People of Color (BIPOC) communities experienced COVID-19 transmission, hospitalization, and death at higher rates than White populations (Saia et al., 2021). Preexisting racial and ethnic disparities were exacerbated by COVID-19, negatively affecting communities of color that tended to be overrepresented in lower socioeconomic groups, had limited access to health care and education, had an undocumented status, and worked in jobs considered "essential," such as food service, maintenance, and transportation (Litam & Hipolito-Delgado, 2021). The toll taken by COVID-19 on the mental health of members of these communities has yet to be fully realized.

Although the COVID-19 pandemic touched the lives of almost everyone in our society (and the world), other events that unfolded simultaneously exacerbated its effect on some communities (Balkin et al., 2021). A second wave of the Black Lives Matter movement occurred concurrently with the pandemic. Black Lives Matter began in 2013 as an expression of hope for Black

people and of outrage over the killing of Trayvon Martin, which occurred in the wake of repeated instances of police violence. At a time when COVID-19 was spreading at a catastrophic rate and causing widespread fear and anxiety, the killings of Ahmaud Aubrey, Breonna Taylor, and George Floyd in 2020 compounded the distress being experienced by Black Americans. The additional trauma and bereavement of Black males, who continue to be disproportionately the victims of police killings in the United States, will continue to need to be addressed by counselors (Lee & Robinson, 2019). The emergence of the second wave of the Black Lives Matter movement has drawn attention to the fact that systemic racism and discrimination affect health outcomes (Balkin et al., 2021) and other domains central to recovery from the COVID-19 pandemic (e.g., employment, education, housing; Edwin & Daniels, 2022; Moreno et al., 2020). Counseling students can begin to learn the skills needed to prevent and resist racial trauma by developing a critical consciousness of anti-Black racism (Mosley et al., 2021) and by participating in advocacy groups.

Members of Asian American and Pacific Islander communities, in particular, experienced violent attacks related to the presumed geographical origin of COVID (Ermis-Demirtas et al., 2022; Litam, 2020; Litam & Oh, 2020). Members of these communities experienced the additional stressors of pandemic-related, race-based prejudice and discrimination (Lund, 2020), which had a negative impact on their mental health. Reports of depression and anxiety symptoms among Asian Americans increased sevenfold during the COVID-19 outbreak (Woo & Jun, 2021).

The pandemic years were particularly challenging for members of the LGBTGEQIAP+ (lesbian, gay, bisexual, transgender, gender expansive, queer, intersex, asexual, pansexual) community. For example, Hawke et al. (2021) concluded that transgender and gender diverse youth were more greatly affected by mental health challenges during the COVID-19 pandemic than cisgender youth. The researchers found that these youth reported significantly more mental health and substance use service disruptions and less social support from their families compared with cisgender youth. Attacks from the political realm were rampant during the COVID-19 pandemic. In April, 2022, for example, the Association for Counselor Education and Supervision (ACES) noted that by March 11 of that year, over 150 pieces of legislation targeting LGBTGEQIAP+ individuals had been introduced in state legislatures across the United States. In response, ACES issued a Unified Statement on Anti-LGBTGEQIAP+ Legislation that affirmed the dedication of ACES members to social justice, advocacy, multicultural content in coursework, antiracist approaches to teaching, and gender-affirming care.

Disruptions in services caused by the pandemic also took a toll on individuals who were living with chronic mental health conditions before the pandemic. Participants in a study by Costa et al. (2021) reported negative impacts on their mental health: 21% reported that their mental health was much worse, and 51% said their mental health was worse. Although the advent of telemental health services enabled many individuals to remain connected with their ongoing mental health providers, individuals diagnosed with some conditions were not as successful in making the transition. Zhu et al. (2022) found that individuals diagnosed with schizophrenia made up a lower proportion of telehealth encounters relative to in-person visits, whereas those with anxiety and fear-related disorders accounted for a higher proportion.

Persons with substance use disorders were a particularly vulnerable population because of their unique social and health care needs, as they faced increased risk for infection with COVID-19 and a severe illness course in addition to ongoing marginalization in health care and social systems (Melamed et al., 2020).

In 2022 and 2023, as pandemic restrictions eased and were lifted, counselors and other mental health professionals began to reflect on some positive outcomes that resulted from the devastating experience of the COVID-19 pandemic, although they also added cautions. Fish and Mittal (2021) lauded state licensing boards for lifting some restrictions on jurisdictional coverage of services so that licensed clinicians could continue to serve current clients and also lauded mental health

providers for transitioning rapidly to teletherapy. These changes inadvertently presented new pathways to accessing mental health services for many populations that might not otherwise have been able to seek or afford care. To the extent that these changes remain in place, they will likely help to narrow gaps in the seeking of mental health care by populations that often struggle to find adequate care. Saia et al. (2021) noted that responses to the pandemic also created new avenues of access for people with disabilities, including telehealth, work-from-home opportunities, and virtual participation. The authors noted that these were the very solutions and accommodations that members of this population had been requesting but had been denied for years. Saia et al. (2021) cautioned against a broad call for a "return to normal," noting that such a call is steeped in ableist thinking. Kopelovich et al. (2021) urged that many of the changes implemented during the pandemic be maintained, including offering a spectrum of options for remote and in-person care, greater integration of behavioral and physical health care, leveraging of natural supports, and integration of digital health interventions.

Components of Multicultural Competence

Cultural disparities that existed in our society were exacerbated by the pandemic and are ongoing. This makes it even more vital that counselors have the competencies needed to serve diverse clients. Multicultural competencies include attitudes and beliefs, knowledge, skills, and actions (Francis & Farrell, 2022). Counselors need to be aware of their own values, biases, and assumptions about human behavior. They also need to be knowledgeable about the cultural values, biases, and assumptions of the diverse groups of clients with whom they work (Gushue, 2022). In addition, they must develop culturally appropriate intervention strategies for assisting these diverse clients. Finally, they must be able to advocate for change at the individual, community, and societal levels.

SELF-AWARENESS A starting point for you in developing cultural self-awareness is to learn how your own cultural and racial identities have an impact on your values and beliefs about the counseling process. Although it may be uncomfortable, you must engage in self-reflection and acknowledge your own biases and assumptions.

For members of dominant groups in the United States, developing multicultural self-awareness takes conscious effort because they have not had to experience themselves as marginalized beings. McIntosh (1998), in a classic article about race and racism, argued that White privilege in our society allows and even encourages a lack of awareness. She described *White privilege* as an invisible package of unearned assets on which White people can rely and about which they are intended to remain oblivious. Her contention is that Whites have been taught not to see this inherited systemic advantage because remaining unaware of its existence maintains the status quo. For example, a White female would be unlikely to fear for her life if she were pulled over by the police for driving with a broken taillight, perhaps without realizing what a different experience that would be for a young Black male.

3-1 The Case of Marlene

Marlene, a White high school counselor, is conducting a group for non–college-bound senior girls. The topic of this session is "preparing for life after graduation." The nine-member group includes equal numbers of girls who are Black and White, and Consuela, who identifies as Latine.

The discussion has turned to a consideration of the advantages and disadvantages of continuing to live at home with parents for a while versus moving into an apartment with friends as soon as one finds a job. Marlene notices that Consuela has remained silent for the past several minutes. In an effort to draw her in, Marlene asks her, "Consuela, isn't it more of an expectation among Latine families that girls will continue to live at home until they marry?"

- What do you think of the question that Marlene posed to Consuela?
- What are some other strategies Marlene might have used to draw Consuela into the conversation?

Discussion: The case of Marlene illustrates how the lack of awareness of one's own assumptions can be manifested in a counseling situation. Marlene had good intentions when she asked Consuela the question to draw her into the conversation. However, she failed to consider how Consuela may have felt about being asked to speak for all Latine families. It hardly seems likely that Marlene would have asked a similar question of one of the White students. In fact, being able to count on not being asked to speak for all members of one's racial or ethnic group is an element of White privilege. Had Marlene examined her unconscious assumptions that may have originated in her own status of White privilege, she would not have used this particular strategy to draw Consuela into the discussion.

KNOWLEDGE In addition to developing cultural self-awareness, you must also seek out knowledge that will enhance your understanding of clients who are culturally different from you (Butler et al., 2022; Lee, 2013). When counselors do not have this knowledge base, therapeutic error is likely to occur. Of course, it would be impossible for any counselor to be completely knowledgeable about the cultural beliefs, values, communication styles, and behaviors of every potential client in our highly diverse society. What is important is that you acquire basic competencies with respect to knowledge of different cultures and that you are open to continuing to learn. A counselor's lack of knowledge can negatively affect the counseling relationship.

3-2 The Case of Jack

Jack, a counselor intern, is working in a college counseling center. Mai-Ling, a freshman who came to the United States from mainland China with her parents 8 years ago, has come for her first session with Jack and has agreed to let him audiotape the session for his supervisor. Mai-Ling states that she has come for help in deciding whether to continue in her 2-year associate degree program in computer operations or switch to a baccalaureate program in computer science. Jack, very much aware that his supervisor will be listening for reflections of feelings and other attending skills, asks Mai-Ling how she feels about being confronted with this decision. She responds by simply restating her problem. The session continues in this fashion, with Jack probing for the client's feelings and the client responding in a cognitive, content-oriented manner. After the session ends, Jack determines to ask his supervisor how to build rapport with this difficult client.

- What does Jack need to know about the culture of persons from mainland China that could have been helpful to him in this counseling session with Mai-Ling?
- What do you think about Jack labeling Mai-Ling as a *difficult client*?

Discussion: If Jack had known that some Asian clients can be very uncomfortable with the idea of discussing their feelings with a stranger, he might not have perceived his client as *difficult*. His lack of understanding led him to make an error that is common among counselors who lack knowledge of different cultures—he blamed the client for his failure to establish a therapeutic alliance.

If you are a student in a CACREP-accredited counselor education program, you will be taking (or may already have taken) a course in multicultural counseling, where you will begin to acquire the knowledge you will need to serve a diverse clientele. You will want to keep up to date by reading books and articles on new developments in multicultural counseling or that focus on specific groups of clients, such as African American youth, and older individuals, to cite just two examples. Specialized training is readily available through workshops and seminars, online courses, and other resources. There is a limit, however, to what can be learned through reading and taking courses and seminars. Experiential learning can be powerful. As you begin to practice, the most important resource for your continued learning will be your clients. They are the most expert source of knowledge about their own cultural values, beliefs, customs, and behaviors, and they are often willing to teach their counselors.

SKILLS The third component of ethical multicultural practice is culturally appropriate skills and intervention strategies for counseling diverse clients. To develop these skills, you must recognize that interventions that are effective when working with the dominant culture may not be effective or appropriate when working with clients who are not White, of European ancestry, and middle class (Bayne & Branco, 2018; Rogers-Sirin, 2017). It is also crucial to realize that many of the traditional approaches to counseling that focus on the internal dynamics of the individual client are not likely to be as useful to clients whose problems originate in societal oppression, discrimination, and marginalization. To serve these clients effectively, you will need to develop a repertoire of helping strategies that consider not only the individual client but also the larger social systems in which the client functions (Butler et al., 2022; Mifsud & Herlihy, 2022). It will be equally vital for you to develop skills in performing alternative roles such as client advocate, consultant, social change agent, and liaison with existing support systems in the client's community (Lee, 2015).

3-3 The Case of Theo

Tameika, a 28-year-old Black woman, comes to the community mental health center seeking counseling. During her initial session with Theo, a novice counselor who is White, Tameika reports several symptoms suggestive of depression, including disturbance in her sleep patterns, loss of appetite, general fatigue, and unexplained weeping spells. Theo inquires as to whether she has experienced any losses or significant life changes recently. She replies, "Not really. I just keep trying to keep it all together. And it's hard when I earn a minimum wage and so much of it goes for childcare for my two little ones. Some months I can't afford both to feed my kids and pay the utility bills. I can't see how it's going to get any better." Theo suggests that she might get some short-term relief by taking an antidepressant medication and offers to refer her to the mental health center's consulting psychiatrist. Tameika agrees to make an appointment, but she never returns to the center.

- What do you think of Theo's suggestions for assisting Tameika with her problems?
- What kind of help does Tameika need?

Discussion: In this case, the counselor would have been more effective had he taken more of a systemic approach to helping Tameika. Rather than focus on her intrapsychic dynamics and her depression, he might have explored with her whether there were any social support systems available in her community. He might have discovered, for instance, that her religious faith was an important source of strength for her, and he could have helped her connect with a church in her neighborhood that offers free or reduced-fee child care for its members. He might have helped her contact social services agencies such as Aid to Families with Dependent Children or a community organization that could help her manage her financial problems. His failure to implement any such culturally appropriate strategies could have been a major factor in Tameika's failure to return to the center.

A skill that has received attention in recent years is broaching, which occurs when a counselor initiates a discussion of racial, ethnic, and cultural (REC) issues that are relevant to the client's presenting concerns (Day-Vines et al., 2020). Day-Vines and her colleagues (2018, 2020) have asserted that because counselors wield the balance of power within the counseling relationship, it is incumbent upon them to initiate discussion of clients' REC concerns. Recently, these scholars offered strategies based on the Multicultural and Social Justice Counseling Competencies for discussing race, ethnicity, and culture with clients (Day-Vines et al., 2021).

ACTIONS Awareness, knowledge, and skills are building blocks to actions that advocate for and with clients. Advocacy is taking actions that will help to remove external and institutional barriers to clients' well-being. As advocates, counselors take actions that affect public opinion, public policy, and legislation, intervening not only in clients' lives but also in the larger social and political arena. Even if you are a beginning student in counseling, you can be an advocate. You can become active in an organization that addresses the societal inequities that matter the most to you, such as a group that advocates for LGBTGEQIAP+ rights or for expanded mental health care for the underserved. You can join with your faculty members and fellow students to spend a day at your state capitol, lobbying legislators to support bills that mandate that more counselors be employed in public schools to meet recommended student/counselor ratios or that ban conversion therapy for minors.

3-4 The Case of Malik

Malik is a counselor in an urban elementary school in an inner-city neighborhood of a large city. He has been counseling Tomas, a fourth-grader who is new to the school. Tomas's parents crossed the border from Mexico to the United States several months ago and are living with relatives while waiting to hear from the immigration authorities as to whether they will be allowed to stay in the United States. The school's student population is diverse, with a majority of Black students and smaller numbers of White students and second-generation Asian students. There are very few Latine students, and none in Tomas's grade in school. Malik's heritage is Black and Asian, so he has

felt comfortable in his knowledge of the students' backgrounds, but he has had no experience with immigrants from Mexico.

Tomas reveals to Malik that he has tried to make friends, but his classmates make fun of his accent and the food in the lunches his mother packs for him to eat at school. He states that he feels left out when his teacher acknowledges Black History month and Asian American Pacific Islander Heritage month. Because Malik realizes that counseling Tomas individually will not suffice to help him feel a sense of belonging at his new school, Malik arranges his schedule so that he can begin offering psychoeducational groups to all the fourth-grade classes, focusing on appreciation for cultural diversity. With Tomas's permission, he also talks with Tomas's teacher, who agrees to create a teaching unit on Hispanic Heritage month and to see if Tomas would be willing to help her develop it.

Discussion: Malik was wise to recognize that Tomas's problems originated in his classroom environment and to take action steps to address the prejudice and exclusion that Tomas was experiencing. He empowered Tomas to advocate for himself by arranging for the teacher to invite him to help plan the teaching unit.

Competence

As is evident, the primary ethical issue that relates to multicultural counseling and advocacy is competence. Although there are some counselors practicing today who remain culturally encapsulated, counseling professionals now generally recognize that they must learn to practice in a diversity-sensitive manner and must develop advocacy skills for intervening in unjust systems (Schuster et al., 2021). Nonetheless, prejudicial attitudes and biases are deeply ingrained in Western society, and counselors, as members of that society, have internalized these biases. Thus, counselors can practice discrimination without being aware that they are doing so (Gushue et al., 2022).

Our own understanding of multiculturalism is based on the premise that *all counseling is cross-cultural*. We define *culture* broadly to include numerous cultural variables, including characteristics into which people are born or that are visible, such as race or ethnicity, social class, gender, age, and disability status. Culture also encompasses characteristics that may not be visible, such as educational attainment, geographic origin, marital status, language, religion, and citizenship status. Ethical issues related to prevalent forms of bias based on these cultural characteristics are discussed in the following subsections, along with strategies to advocate for clients who are members of disenfranchised groups in our society. As advocates, counselors have an ethical responsibility to confront the *-isms* of prejudice and discrimination at the individual, group, and societal levels.

AVOIDING DISCRIMINATION

RACISM Racism refers to prejudice and discrimination against people of color and people of certain ethnic origins. Because the multicultural counseling movement historically has focused on race, a substantial body of professional literature addresses the effects of race and ethnicity on the

counseling relationship, process, and outcomes. When a client and a counselor are racially or ethnically different from each other, counselor insensitivity to the differences can lead to client reluctance to self-disclose, mistrust of the counselor by the client, and premature termination of counseling. Very rarely do counselors behave toward clients in a manner that is deliberately racist. However, even counselors who are strongly committed to multiculturalism are not exempt from deeply embedded racial/ethnic biases and prejudices. Unintended racism can take the form of microaggressions, which are "brief and commonplace daily verbal or behavioral indignities, whether intentional or unintentional, that communicate hostile, derogatory, or negative racial slights and insults" (Sue & Sue, 2013, p. 154) and that have a potentially harmful or unpleasant impact on the target person or group. Ample research evidence exists that microaggressions have a negative impact on the mental health of people of color and persons with disabilities (Kattari, 2020; Nadal et al., 2014; Rivera et al., 2010; Sue et al., 2022; Yeo et al., 2019). Counselors have an obligation to avoid re-creating and reenacting within counseling sessions the dynamics of bias that exist in society.

As advocates, counselors recognize that racial and ethnic discrimination is rooted in the social organization of our society and that mental health is profoundly affected by experiences of marginalization and oppression. Advocacy involves extending the work of counseling beyond the individual client to address the external forces that create barriers to clients' full participation in society. This includes engaging in community empowerment strategies, political action, and other activities aimed at systemic change. Counselors can take the lead in mobilizing the provision of mental health services to communities in need.

SEXISM Sexism and gender issues may be overlooked in counseling. The vast majority of master's-level counseling practitioners are female, and it might be assumed that these practitioners have the awareness and skills to counsel other women effectively. In reality, women and men are equally influenced by gender-role socialization experiences that, if left unexamined, can lead to gender discrimination and incompetent counseling practices.

Counselor self-awareness is an essential starting point to prevent bringing gender bias into the counseling relationship. Self-awareness begins with reflecting on the impact of one's own gender-role socialization experiences. In addition to self-awareness, counselors need to acquire knowledge that is foundational for counseling women, which includes understanding the specific life experiences and worldviews of diverse groups of women. Counselors who work with female clients also should have an understanding of the high-prevalence problems and issues experienced by women, including domestic violence, sexual assault and harassment, body image and objectification, the right to control their bodies, and conflicts between work and family responsibilities.

SOCIOECONOMIC STATUS Socioeconomic status (SES) has been given little attention in the counseling literature, although the link between poverty and mental health problems is clear (Smith et al., 2008). SES affects all dimensions of a person's life, including sense of self-worth, access to mental health care, exposure to violence, and sense of personal power or agency (Robinson-Wood, 2009). Research has demonstrated bias against low-SES clients in mental health services: Clients from low-SES backgrounds are more frequently and more severely diagnosed as having mental health problems, are prescribed different treatments based solely on class, and have fewer services available to them and less experienced clinicians to serve them (Aponte & Wohl, 2000). Counselors themselves may be unclear about the distinctions between SES and social class (social class is more a culture that carries with it group membership, norms, and socialization patterns, while SES includes income status) (Cook et al., 2021), which may contribute to deficiencies in treatment.

A middle-class bias prevails in the training of counselors (Robinson-Wood, 2009). Because most counselors come from or have achieved middle-class status (Bienvenu & Ramsey, 2006), it may be difficult for them to relate to the phenomenon of poverty and to become aware of their class biases (Clark et al., 2017; Havlik et al., 2017). Culturally competent counseling includes the ability to hear class conditioning when listening to clients' stories. For example, a client might be reluctant to take certain occupational risks because of socialization experiences that emphasized security and practicality (Fickling, 2016). Rather than focusing exclusively on personal relationships and insight with a client, counselors should remain aware of and address the social and economic issues that affect the client's life. Counselors must understand the classism that clients may face so that they are able to facilitate client empowerment to cope with the everyday discrimination that these clients encounter (Parker-Barnes et al., 2023).

ABLEISM Ableism is a form of discrimination or prejudice against individuals with physical, developmental, or mental disabilities that is characterized by the belief that these individuals cannot function as full members of society. The number of people with disabilities in the United States is much larger than what is typically assumed. Counselors are subject to the same negative images and stereotypes of people with disabilities (PWD) as the rest of society (Reeve, 2000). This problem may persist because disability has not been widely recognized as a multicultural concern by counselors (Smith et al., 2008), and most counselor education programs (other than rehabilitation counseling programs) have not given much attention to disability issues (Hall, 2015; Hanjorgiris & O'Neill, 2006; Oksuz & Brubaker, 2020). Deroche et al. (2020) found that counseling students may perceive themselves as more competent to work with clients with PWDs if they have disability-related life experience, although completion of a multicultural course was positively related to their perceived competence.

Developing the competencies that are necessary for providing effective counseling services to PWDs begins with counselors' awareness of their own assumptions, values, and biases about disability and with counselors confronting their own ableism. Counselors need to remain aware that not all disabilities are visible; a counselor who is unaware of a hidden disability might erroneously assume that the client's slow progress in counseling is due to client resistance. It is also important to remember that a client's presenting problem may or may not be related to a disability.

Disability issues generally have not been included in social justice agendas (Fabian, 2007), and marginalization is perpetuated when counseling PWDs is assumed to be exclusively the province of rehabilitation counselors. Problems that are related to disability often involve systemic barriers, so counselors should be skilled in advocacy approaches that focus on systems change.

AGEISM Ageism is a growing concern, as the number of older adults in the United States continues to grow (Lum, 2007). Clearly, there will be a burgeoning need for culturally competent counselors to serve this population in coming years. If you expect to work with older adults, you will need to be alert to your own biases. It is important to guard against stereotyping and monolithic thinking or assuming that all older people share the same life issues and counseling concerns.

3-5 The Case of LaTasha

Mary Jo, aged 78, comes to seeLaTasha, a counselor at a community mental health center. When LaTasha asks Mary Jo what brings her to counseling, Mary Jo replies, "I really don't know why I'm here. My daughters pretty much insisted that I see a counselor, and I got tired of their nagging, so here I am." Further exploration elicits the information that Mary Jo's daughters are concerned that

she isn't planning for retirement and that they believe that their mom "should retire and enjoy life while she still can." Mary Jo explains that she has no thoughts of retiring, she loves her work and finds it to be a source of meaning in her life, and her employer wants her to stay on. As Latasha listens, she thinks that perhaps she should inquire more deeply into the sense of meaning that work provides for Mary Jo. Yet LaTasha, who is 28 years old, also finds herself thinking it might be more important to explore whether Mary Jo has considered what she will do if her health starts to fail or if she finds that she is no longer able to perform her job to the satisfaction of her employer.

- What do you think about LaTasha's line of thinking here? If she steers the session toward retirement planning, would she be imposing her own bias about the needs of people who are Mary Jo's age?
- Do you think LaTasha should consider exploring the apparent conflict between Mary Jo and her daughters over Mary Jo's desire to keep working? Would you want to involve the daughters in the counseling process at some point?

Discussion: LaTasha seems to be unaware of the differences in perspective when age 78 is viewed through the lens of a 28-year-old versus the lens of someone who has reached Mary Jo's age. Her lack of awareness of possible age bias may lead her to align with Mary Jo's daughters rather than with her client. This could happen if she focuses on retirement planning rather than the reason Mary Jo has come for counseling, which is that her daughters have insisted and she is feeling nagged. LaTasha might be more effective in working with Mary Jo if she were to stay in the present moment with her client and attend to what Mary Jo has stated, rather than think ahead to what she believes Mary Jo needs to do at this point in her life.

Older adults have tended to underutilize mental health services, perhaps partly because of the stigma they attach to seeking psychological services. Counselors need to be prepared to address the reluctance of some older clients to enter into counseling relationships. Counselors should be alert to signs of possible elder abuse or neglect (see Chapter 11 for further discussion of this topic). As advocates, counselors are uniquely positioned to assist older individuals because our developmental perspective recognizes and supports positive growth across the life span.

DISCRIMINATION BASED ON SEXUAL ORIENTATION Discrimination based on sexual or affectional orientation is an ongoing concern within the counseling profession. LGBTGEQIAP+ individuals have experienced a long history of oppression and stigmatization in the United States. As is true for working with clients who are members of any marginalized group, counselors need to explore their own biases as a starting place for developing their cultural competence. Internalized attitudes of homophobia, homonegativity, and heterosexism among mental health professionals have led to harmful practices, such as assuming that a client is heterosexual, referring to another counselor a client who discloses his or her sexual orientation, acquiescing in the dissolution of a relationship because it is with a person of the same sex, and failing to support the desire for child custody of gay parents. Counselors have become increasingly aware of the need to provide culturally sensitive services to LGBTGEQIAP+ individuals (Dispenza & O'Hara, 2016; Shulman et al., 2017). Counseling Competencies for transgender clients can be found on the ACA website (www.counseling.org).

RELIGIOUS DISCRIMINATION Spiritual and religious bias must be addressed, considering that spirituality and religion play an important part in the lives of many clients (Ivey et al., 2007;

Robinson-Wood, 2009; Steen et al., 2006). *Spirituality* can be defined as a personal relationship with a higher power and faith that may be used to find meaning in life. *Religion* is a shared set of practices and beliefs that is associated with denominational affiliations (Halbert et al., 2007; Robinson-Wood, 2009).

Some counselors are uncomfortable with the idea of addressing spiritual or religious issues within the therapeutic relationship. This discomfort may be due to lack of clarity about their own views on spirituality and religion, a fear of imposing values, concerns about maintaining separation of church and state in school and some other secular settings, or lack of training and questions about competence (Adams et al., 2015; Genia, 2000; Hall et al., 2004; Steen et al., 2006). To assist counselors with integrating social justice advocacy into this arena, Mintert et al. (2020) have applied the Multicultural and Social Justice Counseling Competencies to religion and spirituality.

3-6 The Case of Clarence

Clarence was excited to graduate with his master's degree and immediately secure employment as a school counselor. In his first year, he has enjoyed his work and believes he has been successful in connecting with and assisting the teenage clients who have sought him out. Now, the student in the counseling room with him is Amber, who is 17 and is struggling with issues related to "how far to go" sexually with her boyfriend. Amber states that the pressure to become more active sexually isn't coming from her boyfriend, who is respectful of her wishes, but from her own desire to experience her sexuality. She states that she isn't talking about intercourse and that she is far from thinking about "going all the way." Her struggle is related to her religious beliefs. Her family is active in a church that has very strict rules about premarital sexual activity, and she says she believes in the church's teachings. Clarence considers himself an agnostic, and he is unfamiliar with Amber's church and its teachings. He finds that he is uncomfortable with the idea of exploring Amber's struggle with her religious beliefs and wonders whether he should even be discussing this with her in a public school setting. He steers the conversation away from religion.

- What do you think Clarence should do so that he can be better prepared to continue to work with Amber, assuming she returns for further counseling with him?
- What are the ethical issues involved if Clarence steers the counseling session away from a topic that makes him uncomfortable?

Discussion: In avoiding a topic of discussion that causes him discomfort, Clarence is meeting his own needs at the expense of his client's. He has an ethical obligation to be competent to explore religious issues with his client, and he can learn more about her religious beliefs by asking her about them and how they are creating an internal conflict for her. He will need to keep the focus on Amber's beliefs and set aside any values about sexual behavior that might be associated with his own agnostic religious views. He should seek further training and perhaps supervision so that he can become competent to explore clients' religious and spiritual values when these values are pertinent to the issues that clients bring to counseling.

Because many clients believe spirituality is integrally connected to their personal growth and mental health (Ivey et al., 2005), counselors must be open to discussing client spirituality (Buser et al., 2017). Although it would not be ethical for counselors to impose their own spiritual or religious beliefs and values on a client, counselors should be able to explore spiritual themes when

the client opens a spiritual discussion. LGBTGEQIAP+ clients, in particular, may struggle with the incompatibility of their religious community and their sexual identities (Crockett et al., 2018). Issues that arise when counselors refuse to counsel a client because the client's beliefs or behaviors conflict with the counselors' religious beliefs are discussed later in this chapter.

Intersectionality

Intersectionality is a term used to address the need to understand marginalization based on oppression that moves toward a nuanced understanding of the ways in which interlocking systems of oppression intersect to marginalize individuals in society, rather than on a single-axis framework of one marginalized identity (e.g., race). For many individuals, intersecting systems of oppression create a matrix of domination, which represents the ways that various forms of oppression are structured to produce and sustain inequality (Collins, 2000). Counselors use the concept of intersectionality to explore the ways that systemic, institutional, and structural oppression and privilege impact individuals and communities (Lewis & Grzanka, 2016).

It is important to recognize that individuals have multiple, interrelated cultural characteristics. Considering any of these variables in isolation fails to reflect the complexities of a person's life experiences and worldview (Constantine, 2001, 2002). Constantine suggested that counselors examine how race, class, gender, and other cultural factors interact in their own lives. If you engage in this process, you will recognize that you simultaneously experience both privilege and oppression (as a middle-class White heterosexual woman or as a Christian Black man, for example). Realizing that you occupy both positions on the power spectrum and reflecting on your own experiences of privilege and oppression may increase your understanding and sense of empathy for your clients.

3-7 The Case of Horst

Arthur is a 32-year-old Black man who is employed as a nursing instructor at a community college and is married with two children. He seeks counseling from Horst after he becomes involved in an intimate relationship with Mark, an openly gay man. Mark also works at the community college, and their coworkers have gossiped among themselves about the relationship, although no one has said anything directly to either Arthur or Mark. Arthur believes he has come to the point in his life when heterosexual marriage is no longer viable for him. He presents with several issues, including coming out to his wife and children and redefining himself as a gay man. He believes that his wife suspects the truth and will come to accept who he is, but he fears losing the close relationship he has enjoyed with his children.

- What are the issues Horst must address in providing counseling to Arthur?
- What information will Horst need to be a culturally competent counselor in this situation?
- What skills will Horst need to be a culturally competent counselor in this situation?

Discussion: Horst, as Arthur's counselor, must have awareness, knowledge, and skills across several dimensions if he is to be effective in helping Arthur. First, Horst must be aware of the impact of Arthur's multiple cultural identities. Until recently, Arthur has enjoyed heterosexual privilege, and he may not be prepared to deal with the realities of being an openly gay man. He may find that some of his friends will not be able to accept his gay identity. On the other hand,

Arthur has had experience in coping with a marginalized status—he is Black and works in a predominantly female occupation. Horst must be knowledgeable about sexuality so that he does not make erroneous assumptions, such as believing that Arthur must be bisexual because he has engaged in heterosexual intercourse for many years. If Arthur's wife and children want to join in the counseling process and Horst decides to accept them as counseling clients, Horst must have family counseling skills so that he can help Arthur and his family create new family rules and boundaries. Therefore, he must have a range of competencies—in multicultural counseling, family counseling, and LGBTGEQIAP+ counseling—to be effective in helping Arthur.

Every client, and every counselor, belongs to multiple cultures, and the salience of membership in any of these cultural groups changes according to the situation. At the same time, each individual is unique. You will need to keep your attention balanced in your work with clients. If you pay too much attention to cultural group membership, you may inadvertently stereotype your client. If you focus too narrowly on the client as an individual, you might overlook the impact of the cultural environment on that client (Corey et al., 2010).

With respect to cultural diversity, perhaps more than any other ethical issue, it is crucial that counselors attend to the *spirit* of the *Code of Ethics* (ACA, 2014). Standards regarding nondiscrimination and honoring diversity are included throughout the *Code*, but we believe it would be a mistake to think that only these specific standards are pertinent to multiculturally competent practice. Rather, we believe that *all* of the ethical standards are best seen as multicultural standards. If cultural diversity is a theme that runs throughout the *Code of Ethics* (ACA, 2014), it is also a theme that runs throughout this text. This chapter only introduces the multitude of considerations that are involved in ethically sensitive, multicultural counseling practice. Each of the ensuing chapters focuses on a specific ethical/legal/professional issue, and there is also a section devoted to diversity considerations that relate to that issue. For instance, some diversity issues that arise in informed consent are discussed in Chapter 4. Diversity is far too important an issue in the professional practice of counselors to be limited to one section of this text.

VALUES AND VALUE CONFLICTS

Counselors' Personal and Professional Values

The primary ethical standard related to counselor values is §A.4.b. of the ACA *Code of Ethics* (ACA, 2014), which requires counselors to be aware of—and avoid imposing—their own values, attitudes, beliefs, and behaviors on clients. Although this ethical obligation is easy to state, it can be difficult to uphold in practice. A counselor's values will influence the counseling process in many ways (Levitt & Moorhead, 2013), including which goals are considered appropriate or desirable in counseling, whether and how the client will be diagnosed, and which client issues will become the focus of therapy. Even the theory from which the counselor operates can influence the counseling process in value-laden ways: Certain theories promote specific values. For instance, the Adlerian approach emphasizes the development of social interest as a benchmark of mental health, whereas the existential approach emphasizes individual freedom and responsibility. The influence of counselor values on the counseling process can be very subtle, and counselors can communicate their values in many indirect ways: through nonverbal responses, by focusing on some elements of a client's story while not responding to others, and through the interventions they select (Francis & Dugger, 2014). Counselor values can also significantly affect

counseling outcomes. Research has shown that the degree of congruence between the values of the counselor and those of the client can influence therapeutic outcomes and that clients tend to change in ways that are consistent with the values of their counselors. Jadaszewski (2017) has cautioned against the "values paternalism" (p. 298) that occurs when impositions of counselor values occur outside the awareness of the counselor, which underscores the need for counselors to be aware of their own values.

Both the client and the counselor enter counseling relationships with their own sets of values, which they have acquired through years of experience and exposure. Value conflicts may never become a problem in counseling when the value systems of the client and the counselor are similar or compatible. When the value systems of the counselor and the client differ, however, particularly around emotionally charged issues, these differences can become problematic (Barrett & McWhirter, 2002; Kiselica & Robinson, 2001; Millner & Hanks, 2002). Value-laden issues can create problems if counselors have not clarified how their own personal beliefs might affect their therapeutic work with clients who present with issues that challenge the counselor's beliefs (Guiffrida et al., 2019).

A great deal of controversy has been generated over the ethical issues raised when the beliefs and behaviors of clients are in conflict with the personal values of their counselors. The value conflict that has been at the heart of the controversy has occurred when some conservative Christian counselors and counselors-in-training have refused to counsel LGBTGEQIAP+ clients because these counselors hold the religious belief that non-heterosexual relationships are a sin and must not be condoned through gay-affirmative counseling (Bayne et al., 2021, 2022). This value conflict has generated court cases, legislation, and a substantial body of literature examining whether counselors can use their religious beliefs as the basis for refusing to counsel LGBTGEQIAP+ clients. This conflict also has generated articles expressing a variety of opinions (Giordano et al., 2018; Guiffrida et al., 2019). For example, Smith and Okech (2016a, 2016b) have questioned whether counselor preparation programs in Christian faith–based universities should be eligible for accreditation by CACREP, while Sells and Hagedorn (2016) have argued that counseling students in Christian universities can be taught to prevent imposing their religious beliefs on the clients they counsel.

Because this controversy is ongoing and is situated within a larger movement to deny mental health services to LGBTGEQIAP+ clients, we discuss this issue in detail.

COURT CASES In two court cases, counseling practitioners were terminated from their employment, and in two other cases, counseling students were dismissed from their training programs after they refused to counsel LGBTGEQIAP+ clients regarding same-sex relationships (Bayne et al., 2021, 2022).

In *Bruff v. North Mississippi Health Services, Inc*. (2001), a client asked her employee assistance program (EAP) counselor (Bruff) for help with improving her relationship with her same-sex partner. Bruff explained that "homosexual behavior" was against her religious beliefs and offered to refer the client to another counselor. Although the client accepted the referral, she did not return for counseling and eventually filed a complaint with Bruff's employer. The employer tried to accommodate Bruff's religious beliefs but determined that it would place too great a burden on Bruff's fellow counselors if Bruff were allowed to refer to other counselors all clients whose issues presented a conflict with her religious values. Bruff was placed on leave without pay and was terminated from employment after she refused opportunities to transfer to other positions within the company. Bruff filed suit against her former employer. One of her claims was that her employer had violated her right to freedom from religious discrimination under Title VII of the Civil Rights Act of 1964. Eventually, the Fifth Circuit Court of Appeals ruled against Bruff. The court determined that although employers do have a legal obligation to make reasonable accommodations for religious beliefs, Bruff's *inflexible* position caused undue hardship to her fellow EAP counselors.

A second legal case, *Walden v. Centers for Disease Control and Prevention* (2010), presented issues very similar to those in *Bruff*. In this second case, an employee of the Centers for Disease

Control and Prevention (CDC) sought same-sex relationship counseling from Walden, an EAP counselor. Walden told the client that same-sex relationships conflicted with her religious values and referred the client to another counselor. The client later brought suit, stating that she had felt judged and condemned even though the referral had been satisfactory. Walden's employer tried to accommodate her religious beliefs by asking her to refer future clients without telling them about her religious objections or personal values. When Walden refused, she was laid off and was later terminated after she did not avail herself of opportunities to apply for other positions within the CDC. Like Bruff, Walden filed suit, and the court ruled against Walden due to her rigid position regarding referrals.

Both the *Bruff* and *Walden* rulings were based primarily on issues of employment law rather than on the ethical issues related to refusing to counsel LGBTGEQIAP+ clients regarding their same-sex relationships. Ethical issues related to values conflicts have played a more prominent role in the cases of two counseling students who were dismissed from counselor education programs at Eastern Michigan University and Augusta State University (now Augusta University) in Georgia after they refused to counsel LGBTGEQIAP+ clients because same-sex affectional relationships were not acceptable, acoording to their religious beliefs. Both students sued their universities. The students claimed that the counseling graduate programs violated their constitutional right to express their religious beliefs.

In *Ward v. Wilbanks* (2010), Julea Ward, a master's-degree student in the counseling program at Eastern Michigan University (EMU), was dismissed from the program in 2009 after she refused to counsel an LGBTGEQIAP+ client. Ward was enrolled in her practicum course when she learned that a clinic client she had been assigned to counsel had previously sought counseling for same-sex relationship issues. Ward told her faculty supervisor that she would not be able to counsel this client effectively due to conflicts with her religious beliefs. The supervisor reassigned the client. Later, the faculty held an informal review meeting with Ward to express their concern that Ward was not complying with the counseling program's policies or the ACA *Code of Ethics*, which prohibits discrimination based on sexual orientation (ACA, 2014). The faculty offered Ward the choices of completing a remediation plan, leaving the program, or requesting a formal hearing. After a formal review, Ward was dismissed from the graduate program.

Ward then brought suit against EMU, with the backing of the Alliance Defending Freedom (formerly the Alliance Defense Fund). The U.S. District Court for the Eastern District of Michigan granted a motion made by the university for summary judgment (*Ward v. Wilbanks*, 2010), which means that the judges on the court found that the student did not have a foundation for her lawsuit based on the pleadings that had been filed by both sides (i.e., the claim of discrimination by the student plaintiff and the response by the university). A higher court (U.S. 6th District Court of Appeals) overturned the lower court's summary judgment decision (*Ward v. Wilbanks*, 2012) and remanded the case back to the district court for a jury trial. The suit was settled out of court. According to Rudow (2013), the resolution upheld EMU's program policies and confirmed the right of the program to use the ACA (2014) *Code of Ethics* as the guide for defining ethical behavior.

In the case of *Keeton v. Augusta State University* (2011), student Jennifer Keeton was asked by faculty at Augusta State University (ASU) in Georgia to complete a remediation plan after finishing her first year of the master's-degree program in counseling. In classes, Keeton had often expressed her condemnation of "homosexuality" based on her religious beliefs, and the faculty was concerned that she "might not be able to separate her religious views on sexual morality from her professional counseling responsibilities" (*Keeton v. Augusta State University*, 2011, p. 4). When Keeton failed to complete the remediation plan, she was dismissed from the master's-degree program. She brought suit against the faculty and the university, claiming that her right to freedom from religious discrimination had been violated. The student also asked for a preliminary injunction to prevent ASU officials from dismissing her from the program if she did not complete the remediation plan. The federal district court denied the student's motion for a preliminary injunction, and the student appealed the decision. The U.S. Court of Appeals upheld the lower court's denial of the preliminary injunction.

The particular circumstances surrounding this case are included in the court's written decision. The Court of Appeals concluded that "the evidence in this record does not support Keeton's claim that ASU's officials imposed the remediation plan because of her views on homosexuality. Rather, as the district court found, the evidence shows that the remediation plan was imposed because she expressed an intent to impose her personal religious views on her clients, in violation of the ACA *Code of Ethics*, and that the objective of the remediation plan was to teach her how to effectively counsel LGBTGEQIAP+ clients in accordance with the ACA *Code of Ethics*" (*Keeton v. Anderson-Wiley, et al.*, 2011, p. 17; Rouse & Horn, 2022). The court found that ASU officials did not tell the student that she had to change her religious beliefs, as she had claimed, but instead had required her to separate those beliefs from her work in counseling clients. Keeton's motion for a preliminary injunction to keep the university from dismissing was denied, and her lawsuit against the university claiming that her rights were violated was dismissed by a federal judge in a U.S. District Court in June 2012.

STATE LEGISLATION The issue raised by the students in these two lawsuits has not been confined to the courts; it has been introduced into the legislative arena as well. Legislation that allows mental health professionals to refuse to provide services to LGBTGEQIAP+ clients based on the professionals' religious or personal beliefs is referred to as *conscience clause* legislation. A law has been enacted in Arizona that prohibits a state university from discriminating against or disciplining a student in a counseling or related mental health program because the student "refuses to counsel a client about goals that conflict with the student's sincerely held religious beliefs" (AZ Rev Stat §15-1862, 2011). A similar bill was passed in Mississippi. In Michigan, a similar bill, the Julia Ward Freedom of Conscience bill (HB 5040/SB 518, 2011), contained essentially the same language but was not enacted into law. In Tennessee, another similar bill was introduced. Due to efforts by mental health professionals in that state, a less restrictive version (Tennessee SB 2423/2115, 2014) than the originally proposed bill was enacted that affirms the right to religious freedom but places no restrictions on the ability of counselor educators to fairly educate their students (Wise et al., 2015). To understand the impact of such legislation on those targeted, Grzanka et al. (2019, 2020) explored the perceptions of LGBTGEQIAP+ residents of Tennessee. Not surprisingly, the researchers found that these individuals held a deep skepticism toward mental health care and were aware of the individual and societal consequences of the legislation.

It is concerning that the introduction into state legislatures of these types of bills has been increasing since 2015. In 2022, in response to a wave of legislation targeting LGBTGEQIAP+ individuals, ACES issued a Unified Statement on Anti-LGBTGEQIAP+ legislation, which can be found on the organization's website (https://acesonline.net). In the ACES Statement, it was noted that over 150 pieces of legislation targeting LGBTGEQIAP+ and transgender people had been introduced in state legislatures. The Statement affirmed that it is a basic ethical responsibility of counselors to oppose any efforts to target groups of individuals through oppressive language, acts, or policies.

Values and the ACA *Code of Ethics*

New language in the 2014 ACA *Code of Ethics* clarifies the distinctions between personal and professional values. The revised Preamble spells out the professional values of enhancing human development, honoring diversity, promoting social justice, safeguarding the integrity of the counseling relationship, and practicing with competence and ethical diligence (Francis, 2015). Some counseling students have strongly held religious beliefs and values that cause them to question whether they can provide counseling services to LGBTGEQIAP+ clients or other clients who are members of marginalized groups. As these students progress through their training programs, they will learn how to set aside or *bracket* (Kocet & Herlihy, 2014) their personal values within

the counseling relationship. It is important that these students understand that they are not being asked to change their religious beliefs, only to refrain from bringing these beliefs into their work with clients. Counseling students are expected to embrace the professional values shared by counselors and to set aside their personal values in keeping with the duty to put client welfare first and foremost.

As we have noted, the role of personal values is described in §A.4.b. of the ACA *Code of Ethics*. This standard requires counselors to be aware of—and to avoid imposing—their own values, attitudes, beliefs, and behaviors on clients. Several additional standards clarify related duties. Standard C.5 states that counselors do not condone or engage in discrimination based on clients' membership in marginalized groups, including those whose sexual orientation differs from the orientation of the dominant group.

Some counselors and counselors-in-training who object to counseling LGBTGEQIAP+ clients have argued that they are not competent to provide services to this group of clients. The ACA *Code* makes it clear that counselors are expected to "gain knowledge, personal awareness, and skills pertinent to working with a diverse client population" (§C.2.a.).

Some counselors have argued that they could act in accordance with their religious beliefs and prevent harm by referring LGBTGEQIAP+ clients to other counselors. As support for their argument, they have cited §A.11.a, which states that when counselors lack the competence to be able to assist clients they should avoid entering into or continuing counseling relationships. This standard does not apply, however, to referring an entire class of people, such as LGBTGEQIAP+ individuals. There is a distinct difference between being unable (incompetent) to counsel a client effectively and being unwilling to learn how to do so. Both §C.2.a. (requiring counselors to gain competence in working with diverse client populations) and §C.5 (nondiscrimination) underscore the inappropriateness of referring LGBTGEQIAP+ clients to other counselors. Finally, §A.11.b. states clearly that counselors do not refer clients or prospective clients based solely on the counselor's personally held beliefs or values. These standards are interrelated and need to be understood together. Counselors must understand that they cannot "just go to one statement in the Code . . . and then base their ethical decision making on that" (Kaplan et al., 2017, p. 112).

Before concluding this chapter, we address another ethical issue that can be values-fraught: the counselor's role in end-of-life decision making.

End-of-Life Decision Making

A controversial issue in today's society is whether people who are terminally ill, have an incurable condition, or are in intolerable pain have the right to choose the time and means of their own deaths (Herlihy & Watson, 2004). As more members of U.S. society live to an advanced age and as life-sustaining medical technologies continue to advance, it is likely that counselors will be called on more frequently to counsel clients who are considering hastening their own deaths (Manis & Bodenhorn, 2006). In recognition of this growing need, ACA signed on to an amicus brief submitted to the U.S. Supreme Court in 1997 in support of the role of mental health professionals in assisting individuals with their end-of-life decision making.

A standard in the previous ACA *Code of Ethics* (ACA, 2005, §A.9), acknowledged that not every counselor will feel comfortable or competent to work with terminally ill clients who wish to explore their end-of-life options. This standard, which allowed counselors to refer a terminally ill client who wished to explore end-of-life options to other counselors, based on the counselor's personal values and beliefs regarding the issue (Kaplan & Martz, 2014a), does not appear in the current *Code* (ACA, 2014). Counselors are thus expected to be competent to work with clients

across the entire developmental spectrum, which includes clients who are nearing the end of their lives (Bevacqua & Kurpius, 2013). Patients with terminal illnesses often are ready to engage in end-of-life talk within a supportive therapeutic relationship (Shaw et al., 2017), and counselors need to be prepared to assist these patients. Wheeler and Bertram (2019) emphasize that counselors must know the relevant laws in their states before they enter into counseling relationships with terminally ill clients and their families.

Counseling assistance with end-of-life decision making may have been a particularly valuable service for some older adults during the early months of the COVID-19 pandemic, when waves of the virus struck multiple residents of nursing homes. Because the prognosis was dire for many of these individuals, counseling for potential end-of-life planning may have been beneficial.

Before you work with terminally ill clients, you also will need to explore your own beliefs about end-of-life decision making. Under what circumstances do you think hastening one's own death is an acceptable option to explore? When a person is terminally ill and death is imminent? When the illness is fatal but the person could live for several more years if they undergo multiple, painful surgeries or must take potent medications with severe side effects? When a condition is not terminal but has left a person in constant, intolerable pain? When a terminally ill person wants to hasten their death to avoid being a burden to his family? When the person already has lived well into their 90s? What if the individual is only 30 years old? When you work with clients who want to consider end-of-life options, it is essential that you do not impose your own beliefs and values, even inadvertently, about such issues as living and dying, quality of life, or religion or spirituality. Your task will be to assist clients to make decisions consistent with their own values and belief systems. You should also be prepared to work with family members and be aware that they may need counseling to cope with complicated grief after their family member dies (Crunk, et al., 2017). An excellent resource for learning more about issues surrounding end-of-life decisions is a book entitled *Counseling Clients Near the End of Life: A Practical Guide for Mental Health Professionals* (Werth, 2013).

Clients Who May Be Victims of Illegal Discrimination

Section C.5. of the *Code of Ethics* (ACA, 2014) states that counselors do not condone or engage in discrimination based on "age, culture, disability, ethnicity, race, religion/spirituality, gender, gender identity, sexual orientation, marital/partnership status, language preference, socioeconomic status, immigration status, or any basis proscribed by law." Counselors have a duty to ensure that individuals who are under their care are not harmed (see Chapters 5, 7, and 11). There may be circumstances in which you must take action to ensure that individuals from minority groups are not harmed physically or emotionally by others. For example, McFarland and Dupuis (2001) have argued that professionals have a legal duty to protect LGBTGEQIAP+ clients from violence in schools. Legal duties of counselors related to discrimination are found in the U.S. Constitution as well as laws and statutes.

The U.S. Constitution provided a foundation for making discrimination illegal through the Due Process Clause of the Fifth Amendment (*Adarand Constructors, Inc. v. Pena,* 1995), the Due Process and Equal Protection Clauses of the Fourteenth Amendment (*Truax v. Raich*, 1915), and the First Amendment. In addition, Title VII of the Civil Rights Act of 1964 prohibits public and private employers, labor organizations, and employment agencies from discriminating on the basis of race, color, sex, religion, and national origin. After Title VII was enacted, federal laws were passed that prohibit discrimination on the basis of age (Age Discrimination in Employment Act of 1967) and disability (Title I of the Americans with Disabilities Act of 1990).

The *Code of Ethics* (ACA, 2014) lists the groups that counselors are admonished not to discriminate against and includes groups that are not protected from discrimination by the federal constitution or by federal statutes. The groups that are not legally protected include subgroups based on culture,

ethnicity, race, spirituality, gender identity, sexual orientation, marital status/partnership, language preference, and socioeconomic status. Some states and local government entities, however, have passed nondiscrimination laws for some specific purposes related to these groups (Belton & Avery, 1999). In our view, counselors do not have a professional responsibility to expose those who they think may be illegally discriminating against protected groups or to encourage clients to assert their legal rights or to assert those rights for the clients, which could be disempowering. On the other hand, counselors do have an obligation to assist clients in making decisions about whether to take action, and what action to take, if they are struggling with value conflicts or if they believe their legal rights have been violated, just as counselors would strive to empower clients to make other types of important decisions in their lives.

When clients express concern that they may have been illegally discriminated against, counselors should first help the clients decide whether they want to take any kind of action. Counselors can be very supportive in helping clients to consider their options and the possible consequences, both positive and negative, regarding these options. It is important that counselors stay within their bounds in these situations and that they not render legal advice or promote their own political or moral agendas via the client.

LGBTGEQIAP+ Clients and Family Law Issues

Counselors should be aware of a number of family law issues in contemporary society that may be affecting their LGBTGEQIAP+ clients. Counselors clearly should not provide legal advice to their clients; rather, counselors should refer their clients to attorneys or legal advocacy groups if clients need legal assistance.

Same-sex marriages became legal in the United States in 2015 as a result of the U.S. Supreme Court case *Obergefell v. Hodges*, 576 U.S. 644 (2015); 135 S.Ct. 2584 (2015). In a landmark 5–4 decision, the Supreme Court held that the fundamental right to marry is guaranteed to same-sex couples by both the Due Process Clause and the Equal Protection Clause of the Fourteenth Amendment to the U.S. Constitution. The ruling determined that it is unconstitutional for a state to refuse to give a marriage license to a same-sex couple. Prior to this ruling, same-sex marriages had been legalized in some states, but not in others. Legalized marriage for same-sex couples has revolutionized society in the United States and has dramatically changed the way counselors provide mental health services for same-sex couples and their family members.

Cultural Issues in Crisis Counseling

Culturally competent crisis counseling has never been more needed than in recent years. Much of the work that counselors performed during the COVID-19 pandemic was crisis counseling, as individuals, families, and communities attempted to deal with major disruptions in and threats to their lives. Crisis counselors played a vital role in helping families that were experiencing increased risk of family violence and child abuse during the "stay at home" mandate (Sinko et al., 2022). Litam and Delgado (2020) described specific cultural considerations in crisis management interventions that can be used with clients of color at an appropriate social distance. In addition to the crises precipitated by COVID-19, mass casualty shooting incidents have increased over the past two decades. More than 32 public mass shootings occurred between 2016 and 2020, making it essential that counselors are equipped to treat primary, secondary, and tertiary victims of these incidents (Cowan et al., 2020).

Counselors who assess and manage crisis situations must be sensitive to cultural differences in their work. In discussing models of assessing crisis situations, Myer (2001) and Knezevic and Olson (2014) have cautioned that individuals from various cultures react differently to crisis events. Because nonverbal behavior such as hand gestures and eye contact probably is influenced by culture, Myer has suggested that crisis counselors distinguish physiological responses from gestures.

Myer also has recommended that counselors avoid making stereotypical conclusions regarding nonverbal behaviors such as sitting on one's hands (which could mean the person is cold rather than wants to hide his or her feelings) or lack of eye contact (which could mean the individual is showing respect for the other person rather than displaying a sign of sadness).

Crisis counselors often assess an individual's time orientation to determine whether a client is focused on the past and a loss, the present and a transgression, or the future and a threat (Myer, 2001). Asian Americans may be primarily oriented to the past or present, with little attention paid to the future, and crisis events are likely to be interpreted from a past or present perspective. Native Americans tend to be present-oriented and experience crises in that way.

Myer (2001) has recommended that crisis counselors avoid imposing their values on individuals they counsel in crisis situations. For example, Southeast Asian immigrants may be reluctant to seek assistance from the police when they are victims of crimes because of police oppression in their countries, and LGBTGEQIAP+ victims of domestic violence may refuse to seek help because of fear of ridicule. What may seem to crisis counselors to be self-defeating reactions could have foundations that are culturally appropriate.

Managing a crisis is important to counselors from a legal perspective. Because they may be liable for harm that comes to their clients if a crisis situation is not managed properly, counselors must understand and react appropriately in crises.

Summary and Key Points

This chapter addresses vitally important ethical and legal issues in counseling related to multicultural counseling, social justice, and advocacy. This chapter also explores the related issue of counselor values. The following are key points.

The counseling profession has evolved over the years, from a lack of awareness of cultural differences to a commitment to multiculturally competent practice to an emphasis on social justice and advocacy aimed at improving the lives of all people in our society and across the globe.

- The COVID-19 pandemic had a profoundly negative effect on marginalized persons in our society.
- Multicultural competencies involve counselors becoming self-aware of their biases, values, and assumptions; gaining knowledge about other cultural groups; developing culturally appropriate intervention skills; and taking actions to advocate for and with diverse clientele.
- Multiculturally competent counselors are aware of, avoid perpetuating, and work to eradicate discrimination based on race or ethnicity, SES or social class, ability status, age, sexual orientation, religion, or other group membership.
- All people have multiple cultural identities and belong to cultural groups that are dominant and cultural groups that are peripheralized in U.S. society. Counselors must understand how the intersectionality of cultural identifies affects the client's worldview.

Counselors must be aware of their personal values and should not impose these values on clients.

- Some counselors' religious beliefs have led them to refuse to counsel LGBTGEQIAP+ clients, and these referrals to other counselors have resulted in court cases and state legislation.
- Counselors need to be prepared to assist clients with end-of-life decision making.
- The U.S. Constitution and various statutes prohibit discrimination. When counselors work with clients who believe they have suffered illegal discrimination, counselors can help these clients to consider their options and the possible consequences of those options.
- Counselors need to be aware of family law issues that affect LGBTGEQIAP+ clients.
- Crisis counselors must be sensitive to cultural differences in the ways that clients respond to crises.

Client Welfare and Informed Consent

4

LEARNING OBJECTIVES

4.1 Discuss the primary counselor responsibilities associated with protecting client welfare.

4.2 Describe the rationale for informed consent in counseling.

FOCUS QUESTIONS

1. What steps should counselors take to safeguard the welfare of their clients?
2. What kinds of information do you think clients need in order to be able to give their fully informed consent to enter into a counseling relationship?
3. Under what circumstances do you think it would be acceptable for a counselor to terminate counseling services to a client?

Trust is the cornerstone on which the counseling relationship is built (American Counseling Association, 2014, *Code of Ethics*, Introduction). If clients are to make progress in achieving their goals in counseling, they need to feel safe in the relationship and to trust that the counselor is committed to safeguarding their welfare and respecting their rights. In your work as a counselor, it is fundamental that you put client welfare first and foremost. The counseling profession attaches such importance to this principle that it is the very first ethical standard in the *Code of Ethics* of the American Counseling Association (ACA). Standard A.1.a. states that the "primary responsibility of counselors is to respect the dignity and to promote the welfare of clients" (ACA, 2014). There are very few exceptions to this principle, and exceptions occur only in the rare circumstances when the good of others in society outweighs what might appear to be best for a particular client.

Counselors have what is known in law as *fiduciary relationships* with their clients. A fiduciary relationship means that the counselor promises to act in the best interests of the client, and the client accepts that trust (Wheeler & Bertram, 2019). The duties of fiduciaries are so strong that any transaction between a fiduciary and the recipient of services is presumed to be fraudulent and void if the fiduciary benefits from it. Transactions that benefit a fiduciary (or counselor) will be upheld in court only when the counselor can present clear and convincing proof that the transaction was legitimate (Ludes & Gilbert, 1998). Because of the law related to fiduciaries, counselors have a significant legal burden to protect the best interests of their clients and to avoid any interactions or transactions with clients that benefit the counselors themselves at the client's expense.

The Impact of the COVID-19 Pandemic on Client Welfare and Informed Consent

Massive numbers of individuals and families suffered multiple losses (of loved ones, employment, and their routines of daily living) during the COVID-19 pandemic, while simultaneously losing the ability to receive in-person counseling services. This resulted in a dramatic increase in virtual counseling, which has continued after individuals have largely returned to their pre-pandemic lives. As counselors have shifted to providing online counseling services, they have needed to adapt their practices to ensure that they continue to promote and protect client welfare.

The COVID-19 pandemic presented challenges to protecting client welfare with respect to interruption of services, termination of client sessions, and abandonment of clients. Virtual counseling relies on technology, which sometimes does not work properly. Counselors thus need to have backup plans if their virtual counseling platform is not working when counseling sessions are scheduled. Alternatives such as telephone calls, texting, or email communications should be considered for connecting with clients and rescheduling virtual sessions that are not possible due to the failure of technology.

When providing counseling virtually, counselors should continue to secure informed consent to ensure that clients understand what counseling entails before they agree to participate in the process. An informed consent discussion can be held over the internet, but a signed document also needs to be obtained. Electronic signatures are now legal in the United States. In 2000, a federal law was passed stating that electronic signatures are legal in every state and U.S. territory where federal laws apply. The legislation that was enacted is known as the U.S. Electronic Signatures in Global and National Commerce Act (15 U.S. Code Chapter 96), also known as the E-Sign Act. Acceptable e-sign signatures include a name typed onto a signature block, a scanned or digitized image of a handwritten signature attached to an electronic record, or a handwritten signature input onto an electronic signature pad.

CLIENT WELFARE

Protecting client welfare is the foremost ethical obligation of counselors. Counselors have a number of specific responsibilities related to safeguarding client welfare. These responsibilities begin when a counseling relationship is entered and extend through termination of the relationship.

Counselors must remain aware of their own needs and motivations when they are working with clients. As is discussed in Chapter 3, they also must be aware of and able to *bracket*, or set aside, their personal values. In addition, counselors must work to safeguard client welfare by preventing the fostering of dependent relationships, using appropriate counseling techniques, and managing any interruptions that might occur during the counseling process. They should also terminate counseling relationships in an appropriate manner and without abandoning clients who need continued services. Mandated or involuntary clients have certain rights that counselors need to safeguard as well.

Counselor Needs and Motivations

It is vitally important for counselors to be aware of their own needs and motivations in order to maintain a fiduciary relationship that keeps client welfare foremost. Without this self-awareness, counselors run the risk of meeting their own needs at the expense of the client's needs.

4-1 The Case of Lynn

Lynn has been counseling Elaine, a 30-year-old client. During this session, Elaine states that she wishes she could just walk away from her marriage, but she can't do it because it would traumatize her children. Lynn, herself a child of divorce whose father abandoned the family, further explores Elaine's fears for the children. At one point Lynn says to her client, "Well, yes, the statistics do show that a lot of kids lose touch with their fathers after a divorce. It would be really hard for them if that happened. It could even have repercussions well into their adult years."

- What do you think motivated Lynn to respond the way she did?
- Do you see any problem with Lynn's response?

Discussion: It appears that the counselor, Lynn, is interjecting her own feelings about paternal abandonment into the session. Lynn is focusing on her own feelings rather than trying to help Elaine clarify her own feelings about the issue. By focusing on the client's children rather than on the client herself, Lynn is discounting the client's feelings and imposing her own values about the effects of divorce on children onto the client.

This case example illustrates that, if counselors lack self-awareness of their *unfinished business*, areas of vulnerability, and defenses, there is a risk that they will inadvertently be working on their own issues in the counseling relationship rather than on the client's issues. As Corey et al. (2024) have noted, counselors who are unaware of their own issues are in a poor position to recognize the ways in which their personal lives are influencing their interactions with clients. This is particularly true when the client's problems or issues are similar to the unresolved issues of the counselor.

We are not suggesting that counselors do not have their own needs or that it is unethical to get some of these needs met by choosing counseling as a profession. In fact, most counselors would probably say that they get a real sense of satisfaction or fulfillment from their work and from knowing that they have made a difference in the lives of others. Still, it is important for you to explore your own answers to these questions:

"What motivates me to choose the counseling profession?"

"What do I get out of being a counselor?"

"What needs of mine am I getting met?"

Knowing your own needs will help you identify potential areas of vulnerability and sources of therapeutic error.

For example, if you have a strong need to nurture others, you may be tempted to encourage client dependency. You might also encourage client dependency if you have a strong need to feel important or even indispensable to your clients. If you have a strong need to be liked and appreciated, you may avoid confronting your clients. If you have strong needs to feel useful or to prove your competence, you may want to rush in and give answers or advice to your clients. If you can recognize your potential areas of vulnerability, you will be better able to keep the best interests of your clients foremost and less likely to allow your needs to intrude into your work with clients. You will be better able to avoid actions that meet your personal needs at the expense of your clients' needs.

When counselors project their emotions onto a client and the counselors' own needs become entangled in the therapeutic relationship, this phenomenon is commonly referred to as *countertransference*. Countertransference can benefit a counseling relationship if counselors are aware of and can monitor their internal reactions to a client and how those reactions are influencing the counseling process (Corey et al., 2024), but countertransference becomes problematic when counselors lack self-awareness and lose their objectivity. What is important to remember is that sound ethical practice requires counselors to pay attention to their emotional reactions to clients and to seek consultation or supervision when these emotional reactions are interfering with the work that is occurring in the counseling relationship.

It is also important for you to keep in mind that counselors, by the very nature of the relationship between a help seeker and a help provider, have power over their clients. Because professionals possess knowledge and skills beyond those of the ordinary citizen, our society extends privileges to professionals. Individuals who seek services from professionals must trust those professionals to practice in a manner that is helpful, and not harmful, to them. With respect to the counseling relationship, clients may not know whether counselors are treating them in a way that benefits them or are taking advantage of them or their lack of knowledge of how the counseling process is supposed to proceed. Counselors could use the power they have in professional counseling relationships to meet their own personal needs rather than the needs of the client. As a member of the counseling profession, you have an ethical obligation to avoid misusing your power and to keep the needs of your clients foremost in the therapeutic relationship.

Preventing Dependent Relationships

Counselors must avoid fostering dependent counseling relationships with clients. Clients have the right to expect that the counseling process will help them learn new skills for living and develop an increased ability to make decisions without the counselor's help. We have mentioned the possibility that some counselors might encourage client dependency on them. This could happen to you in a number of ways if you are not alert to the potential problem of fostering client dependency. During your internship, you must complete a certain number of contact hours with clients, and you might be tempted to keep some clients in counseling longer than necessary in order to fulfill your requirement. You also might do so in hopes of demonstrating to your supervisors that you are so helpful to your clients that they keep returning to see you. As was noted earlier, temptations to foster dependency might arise out of a need to feel needed, a need to nurture others, or a need to feel that you are important or indispensable to your clients. After you graduate, if you are in private practice, you might be tempted to unnecessarily prolong counseling out of a need for income from client fees.

Sometimes it is not the counselor's needs but rather the client's needs that work to foster a dependent relationship. Some clients may attempt to maintain or even increase their dependency on the counselor. They may request more frequent sessions, bring up a new crisis every time the counselor suggests that they may be nearing the end of their work together, or engage in other strategies to prevent making progress toward independent functioning. For these clients, remaining dependent on the counselor is less risky than learning to live autonomously. Counselors must therefore find ways to balance ongoing support for them while encouraging independence and risk taking.

The dramatic increase in the number of clients who participate in managed care health plans has had an impact on this issue. Managed care plans limit clients to a certain number of sessions and require counselors to justify any requests for additional sessions. Thus, it is much more difficult for counselors to unnecessarily prolong a counseling relationship. Nonetheless, counselors are obligated to work toward helping clients reach a place where they no longer need counseling and should be motivated by an internalized sense of responsibility to promote client welfare.

DIVERSITY CONSIDERATIONS When counselors work to uphold their ethical obligations to prevent fostering dependency and to promote client independence and autonomy, they sometimes misapply the principle of autonomy. Counselors need to keep in mind that autonomy is a highly individualistic concept and that the counseling goal of having clients achieve individual independence or autonomy may not be appropriate for clients whose choices are made in the context of family, group, or community (Herlihy et al., 2018; Mifsud & Herlihy, 2022). With the easing of COVID travel restrictions, counselors are again becoming engaged in trauma work internationally. They need to realize that Western concepts of autonomy may conflict with the needs of refugees (Bemak & Chung, 2017) or victims of mass trauma or humanitarian crises (Tarvydas et al., 2017). At the same time, counselors must not lose sight of the facts that there are within-group differences and that each client is unique. Counselors working globally need to remember that many cultures in the world are collectivistic rather than individualistic in nature. In these contexts, a counseling goal of promoting a client's individual autonomy would be culturally inappropriate and would likely even be harmful.

Mandated Clients

Special ethical considerations exist when counselors are assigned to work with reluctant clients (Borsari & Tevyaw, 2005; Kiracofe & Wells, 2007). In some situations, clients may attend counseling sessions because someone else has pushed them in that direction even though the clients themselves are not convinced that counseling is a good idea for them. In other, more serious circumstances, clients may have been ordered to attend counseling or else suffer negative consequences. Sometimes judges require individuals to attend counseling or go to jail.

The term *involuntary client* probably is not the best pairing of words. A client who chooses to enter into counseling rather than accept an alternative or an unwanted consequence is, in fact, making a choice. Mandated clients do choose counseling, however reluctantly, over other options (such as jail). The ACA *Code of Ethics* (2014) supports this reasoning in §A.2.e., which states that mandated clients may refuse services. This standard also states that counselors are ethically obligated to explain, to the best of their ability, the potential consequences of such refusal (§A.2.e.).

The term *mandated clients* usually refers to clients who are involved in the criminal justice system and are receiving counseling services as a condition of their release or progression through the system. These clients usually are required to sign a waiver of their privacy rights when they begin a counseling relationship. Because they are being required to participate in counseling and would suffer negative consequences if they refused, the person or entity requiring the counseling also requires some type of report regarding their participation in the counseling process. Counselors who work within the criminal justice system must balance this reporting requirement against their desire for clients to feel safe to do therapeutic work on the multiple traumas they face, as many offenders have a preexisting trauma history and regularly experience traumatic events while incarcerated (Pringer & Wagner, 2020). Thus, it is crucial that counselors work to promote a safe context by providing structure and guidelines for safety and confidentiality (Sotero et al., 2016) and by ensuring that the informed consent process includes a thorough explanation of the consequences of the choices that are available in the circumstances.

The legal issue involved in these situations is to ensure that involuntary clients know that reports must be made and understand the types of information that will be included in the reports. The privacy waiver form that involuntary clients sign should contain as much specific information as possible. Because different individuals and entities require that various types of information be reported, the form should allow for detailed information to be added. A model agreement form for involuntary clients that includes a waiver of privacy is included in Appendix B.

An ACA *Code of Ethics* provision that applies to counseling involuntary or mandated clients states that counselors nust be accurate, honest, and objective in reporting their professional activities and judgments to third parties, including courts or others who receive evaluation reports (ACA, 2014, §C.6.b.). Although counselors who work within the criminal justice system cannot avoid making reports, when possible it is best to view counseling and evaluation as two separate processes and for counselors who counsel involuntary clients to refuse to evaluate those same clients. Reporting on counseling attendance and progress toward reaching counseling goals set by clients is different from evaluating clients for issues such as fitness for parenting, ability to work after an injury, a mental condition, or progress toward eligibility for parole.

Sometimes counselors do perform such evaluations, but counselors who evaluate individuals must be unbiased. If they have counseled a client in the past, they are biased and should not agree to evaluate that same person. If a counselor is evaluating a client, the counselor must make clear to the client that the purpose of their meetings is to gather information for an evaluation, not to provide counseling services. Further, clients must understand that there is no confidentiality in such situations and that their counselors will report their findings to a third party. See Chapter 7 regarding the requirements of counselors who serve as evaluators.

The ACA *Code of Ethics* (2014) addresses role changes in the professional counseling relationship. When counselors change from an evaluative role to a therapeutic role, or vice versa, they must obtain the client's informed consent for the change and explain the client's right to refuse services related to the change (§A.6.d.). Counselors must be particularly sensitive to the rights of reluctant, involuntary, or mandated clients. It is essential that counselors fully disclose to such clients any privacy rights that clients are waiving as they enter into such arrangements. For example, if a judge who orders an individual to engage in counseling requires periodic reports to a court official (such as a probation officer) on the person's attendance and progress toward counseling goals, counselors must obtain written permission from the client for such reports and must ensure that the client has a full understanding of the types of reports that will be made. Granello and Hanna (2003) have suggested that counselors who provide services to individuals involved with the criminal legal system must develop a good understanding of the system to serve their clients well.

Counseling Techniques

Another consideration in promoting client welfare is the counselor's obligation to select appropriate counseling strategies or techniques. In recent years, mental health professionals have worked to develop guidelines for treatment planning that match client concerns with the strategies that research has demonstrated to be the most effective in treating those concerns. Hansen (2005, 2006) has expressed a concern that the movement in the counseling field in which a particular approach is required to treat a particular presenting problem takes away the essence of counseling, which is human beings connecting in a creative, meaningful, and therapeutic manner. Despite such cautions, the ACA *Code of Ethics* requires counselors to use techniques, procedures, and modalities that are grounded in theory and/or have an empirical or scientific foundation (ACA, 2014, §C.7.a.).

If you use any techniques that could be considered experimental or unusual, you should do so very cautiously (Ho, et al., 2012). The client should be informed of any potential risks and benefits

and should consent to your use of such techniques. Before attempting these techniques, you should be sure that you are adequately trained and have a sound clinical rationale for their use. For example, as suggested by Crockett et al. (2017), counselors who choose to use techniques such as neurofeedback in their practices must understand the procedures to use and be properly prepared to use those procedures. Although neuroscience is a burgeoning specialty area of practice, concerns remain about the potential for client exploitation if the informed consent process misleads clients into holding unrealistic expectations of the techniques (Luke et al., 2020). Before employing an experimental or unusual counseling technique, think about how you would justify your procedures if a complaint were filed against you by a licensing board or in a court of law.

As a counselor, your selection of techniques will depend on a number of factors, including your theoretical orientation and your training. Corey et al. (2024) put it best when they advised that your strategies should fit your counseling style and be tailored to the specific needs of your client. Of course, counselors should not use techniques that might cause harm to a client. One example of such a technique is conversion therapy or reparative therapy, which entails attempting to change a client's sexual orientation to align with heterosexual norms. In 1999, the ACA Governing Council adopted a statement opposing the promotion of reparative therapy. In 2006 the ACA Ethics Committee issued an interpretation of the ACA *Code of Ethics* regarding conversion therapy and the ethics of referring clients for this practice. In the current ACA *Code of Ethics* (2014), a standard (§C.7.c.) specifies that counselors should not use techniques, procedures, or modalities when substantial evidence suggests harm from using these techniques, even if clients request those techniques. Researchers have demonstrated that reparative therapy is detrimental and causes damage to clients (ALGTBIC, 2009; Conine et al., 2021; Finnerty et al., 2017) and should not be used or condoned.

Counselors have an ethical responsibility to work jointly with clients to devise counseling plans that offer a reasonable promise of success (ACA, 2014, §A.1.c.). As we have mentioned, managed care organizations typically limit the number of sessions for which a client will be reimbursed and require counselors to justify any requests for additional sessions. Counselors will be more successful in making such requests when they can present empirical support for their clinical judgments.

DIVERSITY CONSIDERATIONS Counselors who espouse traditional counseling theories and their associated techniques must be aware that these approaches have limited applicability in working with culturally diverse clients (Jencius & West, 2003).

Psychodynamically oriented counselors (Neo-Freudians) who focus on their clients' early childhood relationships with their parents and on intrapsychic conflicts may overlook a client's historic experiences with discrimination and oppression. A psychodynamic therapist's strategy of remaining relatively anonymous to serve as a projection screen for the client's transferences could be a poor fit for clients from cultures that value cooperative relationships or for clients who expect the helper to take an active role.

Cognitive behavioral therapists are more active in implementing strategies such as direct teaching of skills and assigning homework. However, some techniques, such as identifying cognitive distortions or irrational self-talk, fail to consider the language-bound nature of cognitions. Cognitive behavioral approaches, like psychodynamic approaches, may not incorporate variables such as race, ethnicity, class, gender, or culture that may be salient to the client's core identity. From the client's perspective, the therapeutic alliance has been found to be positively related to the counselor's multicultural orientation and multiculturally competent behavior (Oh et al., 2022).

Humanistic counselors, particularly those with a Rogerian orientation, offer clients acceptance, unconditional positive regard, and empathy, all of which may help to transcend cultural differences and establish a therapeutic alliance. However, these approaches may fail to consider that barriers to self-actualization may exist in the client's environment rather than within the self. In addition, the nondirective stance may not meet the needs of clients who might benefit from advocacy and assistance in navigating an oppressive culture.

Today's counselors must have repertoires of helping strategies that extend beyond those offered by traditional counseling theories (nearly all of which were created by and reflect the worldviews of Euro-American White males). It is particularly important for counselors, as they step into the international arena, to have training in a wide variety of counseling techniques, including those that originated in non-Western cultures, and to be competent in working in nontraditional roles.

Interruptions and Termination

In an ideal world, once a counselor has accepted a client and begins providing services to that client, those services will continue on a regular basis until the client is able to function effectively without the ongoing assistance of the counselor. In reality, however, counselors must sometimes interrupt counseling services for a period of time, and counselors occasionally terminate counseling relationships before clients are prepared to discontinue counseling services. Sometimes, clients will terminate counseling prematurely, for various reasons. From both an ethical and a legal perspective, counselors must be careful to protect the best interests of clients when services have to be interrupted or are prematurely terminated.

INTERRUPTIONS IN SERVICES There is truth in the wry observation of Pope and Vasquez (2016) that both counselors and clients tend to find comfort in the fantasy that the counselor is invulnerable. The reality is, however, that sometimes counselors fall seriously ill, have accidents, or are called away by family emergencies. Counselors must therefore plan ahead for emergency absences by developing procedures to safeguard their clients' welfare. Anticipating that unexpected events might occur, counselors should have plans in place regarding who will notify the clients, who has access to client records, and how the transfer of clients to another counselor will be handled.

Counselors are also no different from other workers in that they take vacations, go out of town to attend conferences, and do other things that cause interruptions in the weekly rhythm of the therapeutic relationship. Clients sometimes have strong and even puzzling reactions to their counselor's absence, so it is important that counselors give clients adequate notice and ensure that clients know whom to contact in a crisis or emergency. The ACA *Code of Ethics* requires counselors to make appropriate arrangements for the continuation of treatment during interruptions caused by vacations or illnesses and, when necessary, following termination (ACA, 2014, §A.12).

Counselors who counsel clients at risk for harm to self or others are well advised to arrange for those clients to also be under the care of other professionals who have accepted responsibility for the well-being of the clients. Counselors should work to ensure that all clients are able to tolerate counselor absences for reasonable periods of time.

TERMINATION Termination is an important stage in the therapeutic relationship that sometimes is given inadequate consideration. Termination is an intentional process that occurs over time when a client has achieved most of the treatment goals, and it provides opportunities for consolidating, making plans for maintaining, and generalizing client gains made in counseling. Effective terminations model how healthy relationships should end and provide clients with opportunities to work through their feelings about ending their therapeutic journeys.

Proper termination of the counseling relationship conveys caring and helps to prevent harm, which are important ethical considerations (Corey et al., 2024; Vasquez et al., 2008). Clients should be able to expect that their counseling sessions will end when they have received what they were seeking from counseling or when they have realized the maximum benefit from the counseling services. Counselors should always anticipate termination as a phase or stage in the counseling relationship, and they should provide pre-termination counseling or recommend other service providers when necessary (ACA, 2014, §A.11.c.). In addition, counselors should raise and discuss the issue of termination with the client well in advance of the final session. Doing so allows ample time to plan for the client's transition to functioning without the counselor and to dealing with the natural and appropriate issues of separation and loss. It is best if the counselor and client mutually agree on the timing of the termination. However, depending on the circumstances, the ultimate decision might be made independently by either the counselor or the client.

PREMATURE TERMINATION Not all counseling relationships end smoothly and by mutual agreement. In reality, circumstances can arise that allow the counselor to terminate the counseling relationship even against a client's wishes. According to the ACA *Code of Ethics* (ACA, 2014, §A.11.c.), it is ethically permissible for counselors to terminate counseling relationships in the following circumstances:

- It is apparent that the client no longer needs assistance, is not likely to benefit, or is being harmed by continued counseling.
- Clients do not pay the fees as agreed upon.
- The counselor is in jeopardy of being harmed by the client or someone with whom the client has a relationship.

When counselors are unable to be of continued assistance to clients or decide to end the relationship over the client's objection, counselors must give adequate notice and give the client time to secure another mental health care provider (Wheeler & Bertram, 2019). In addition, counselors should suggest an appropriate referral and do all they can to facilitate a smooth transition to the new mental health professional. If the client is unable to continue to pay the counselor's fee, an appropriate referral can be made to free services or to an agency that uses a sliding fee scale, if such alternatives are available. If a client refuses to accept the suggested referral, the counselor is not obligated to provide free services.

Many clients are able to afford counseling services only because they have managed health care plans that will reimburse them for a significant portion of the cost. Under managed care plans, people are given only the services that the managed care company deems to be necessary and appropriate through a process known as *utilization review* (see Chapter 13 for a detailed discussion of health care plans). Counselors face difficult ethical challenges when further treatment is denied by utilization review panels. Counselors must maintain clinical responsibility for the clients' welfare, and when clients need further counseling, counselors cannot allow financially motivated decisions to supersede their clinical judgment. Counselors should thus appeal, or have their clients appeal, utilization review decisions when they believe clients are in need of continued care.

Sometimes it is the client who terminates the counseling relationship before an agreed-upon termination. Swift et al. (2012) have noted that approximately 20% of clients drop out of counseling prematurely. Even when the counselor believes that a client is making progress and that treatment goals are being met, clients may call and cancel with or without giving an explanation, fail to keep appointments, or fail to reschedule. The counselor's ethical obligation in these situations is to attempt to communicate a willingness to continue to meet with the client, either to continue counseling or to summarize and end treatment, or to refer the client to another mental health professional. Wheeler and Bertram (2019) have noted that legal difficulties can occur because of these no-closure terminations, and they advise counselors to document carefully and to send a letter to the client indicating that the client's apparent decision to terminate has been accepted.

Both client and counselor characteristics can contribute to the risk of terminating counseling services prematurely. Anderson et al. (2019) found that client characteristics of being female and identifying as a member of a sexual minority were associated with an increased risk of premature termination and that clients were more likely to terminate services when they perceived their counselor as lacking multicultural competence. There is also some evidence to suggest that when counselors are ethnically or linguistically different from their clients, clients are more likely to terminate prematurely (Maramba & Nagayama Hall, 2002). These findings underscore the importance of counselor sensitivity to the experiences of disempowered members of society and of developing and maintaining multicultural competence, as discussed in Chapter 3.

Preventing Abandonment

Abandonment can be considered a form of inappropriate termination and can occur when the client's counseling needs are not being adequately addressed by the counselor, either when treatment ends or during the course of treatment due to counselor unavailability. Ethical guidelines in the ACA *Code of Ethics* (ACA, 2014, §A.12) and legal decisions related to physicians (*Allison v. Patel*, 1993; *Manno v. McIntosh,*1994) prohibit counselors from abandoning their clients. Once a counseling relationship has been established, the counselor cannot arbitrarily end it if the client will be put at risk as a result. Terminating during a client crisis or emergency could put the client at risk and could result in ethical and legal difficulties for the counselor (Wheeler & Bertram, 2019).

As noted, advance planning can help avoid the risk that clients will feel abandoned by their counselors. Counselors should take care to give clients ample advance notice when planned interruptions are anticipated, such as the counselor's taking maternity or paternity leave, undergoing a surgical procedure, or going on vacation.

The key to ending a professional relationship with a client properly, and thus not being exposed to charges of abandonment, is to give the client adequate notice that you are ending the professional relationship and ample opportunity to locate a new counselor if needed. It might be important, too, to discuss with the client how to locate agencies and individuals who offer counseling services. Giving notice of termination and time for the client to find a new counselor may not, however, lead to the client taking the initiative to find a new counselor. If a client refuses to see another counselor after you have given notice that you are withdrawing your services, it will be essential for you to document carefully and fully the notice you gave, your assistance with the referral process, and the time you gave the client to locate a new professional.

Hilliard (1998), an attorney who represents mental health professionals who are sued by their clients, has observed the events that lead individuals to sue their mental health providers. As a practical consideration, Hilliard has recommended that mental health care providers never terminate a professional relationship over a client's objection when the client is angry or when the client is in a serious crisis situation. Taking the time to address a client's distress is very important to prevent unresolved angry feelings that could lead to a lawsuit. Also, a judge or jury probably would not support a mental health professional's decision to end a relationship in the midst of a crisis (*Allison v. Patel*, 1993; *Manno v. McIntosh*, 1994). Ending a counseling relationship because insurance benefits have run out, even as the client is at risk for suicide, is very risky (Packman & Harris, 1998) for both the client and the counselor.

Of course, counselors may always refer a client to a psychiatrist or a health care facility that evaluates individuals for suicide risk or danger to themselves or others. This referral does not always lead to a transfer of that client to another professional or termination. In many circumstances, counselors continue to provide services to a referred client in collaboration with another mental health professional. When such a referral is made, however, it is very important that the counselor inform

all parties whether the referral is a transfer of the client to another professional or the counselor will be providing services in conjunction with the other professional.

To protect mental health professionals from being held accountable for abandonment of their clients, Hilliard (1998) and Macbeth et al. (1994) developed the following guidelines for mental health professionals to use when they are terminating their relationships with clients in adverse circumstances:

- Honestly discuss with the client your intention to terminate and the specific reason for your decision.
- Give the client an opportunity to make changes that would be necessary to continue the relationship.
- Give written notice to the client that the relationship will be terminated.
- Offer the client one to three termination sessions.
- Give your client up to three good referrals.
- Give your client places to contact in the event of emergencies.
- Place in the client's file a summary of your interactions with the client that are related to the termination issue. Do not transfer that document to another individual or entity without an express written request from the client.
- Give the client plenty of time to find another mental health professional. If more time is requested, allow it.
- Transfer records to the new mental health professional promptly.

4-2 The Case of Benita

Although Benita has been working with her client Julianna for 3 months, they have actually met for only 8 of their 12 scheduled weekly sessions. Twice, Julianna failed to appear for her appointment and later said that she forgot. On two other occasions, Julianna called to cancel a day or two in advance and left a message with no explanation. During the counseling sessions, Julianna has been apologetic about the missed appointments and has been open about her struggles with trying to be more assertive in her personal and work relationships; however, she has not completed any of the assertiveness-training exercises that Benita has suggested as homework. Today, Julianna leaves a message that she doesn't think counseling is doing her any good and that she doesn't intend to continue the process.

Benita has to admit to herself that she feels relieved. She sometimes had felt annoyed with Julianna for cancelling or not showing up for sessions and for failing to even try the homework assignments. Also, Julianna's lack of progress had led Benita to question her own effectiveness as a counselor. Benita decides to send Julianna a brief letter acknowledging the termination of their counseling relationship and offering to provide her with a list of referrals if she later decides to resume counseling.

- What do you think about the way Benita has decided to respond to Julianna's message?
- If you were to advise Benita to self-reflect on her work with Julianna, what might you want her to consider?

Discussion: Benita's intended letter to Julianna may meet the rudimentary intent of the ethical requirements, but it falls short of a response that would reflect best practice. We would advise Benita to attempt to contact Julianna to schedule a termination session in which Benita should encourage Julianna to give her honest feedback about the counseling process as she has experienced it.

Benita seems to be blaming herself for Julianna's lack of progress and to be reacting out of her relief that she will no longer need to question her counseling efficacy. If Benita can arrange to meet with Julianna again, even if the client remains steadfast in her decision to terminate their sessions, Benita can convey the message that perhaps Julianna isn't quite ready to take action toward change but that she is welcome to return at a later time or contact Benita for a referral. Together, they can also review whether other factors may have contributed to the lack of success: Perhaps the goal should have been revisited, or maybe Julianna didn't feel safe in the relationship. If Julianna isn't willing to meet again, we would advise Benita to look at some sample letters to see how she might convey in her letter her acceptance of both the decision and the client.

DIVERSITY CONSIDERATIONS As we have noted, clients sometimes drop out of counseling without discussing with the counselor their intentions for doing so. This can happen for many reasons—sometimes clients just need to *take a break* from the intense personal explorations involved in counseling, or they may drop out or discontinue the counseling relationship because they do not believe they are benefiting from counseling. Client termination of the relationship will occur in any counselor's practice, and it may be helpful to remember that no counselor is skilled enough to be able to assist every client. Thus, it is important that you self-monitor to discern whether the clients who are discontinuing counseling with you are those who are culturally different from you. If you see such a pattern, this is a signal that you must work to increase your cross-cultural counseling skills.

INFORMED CONSENT

Informed consent is such a vital and complex component of protecting client welfare that it is discussed as a distinct factor in this chapter. Securing the client's informed consent is both a legal and an ethical obligation. From an ethical perspective, the rationale for informed consent is simple: Clients have the right to know what they are undertaking when they come for counseling. Most clients are not experts in counseling, so they must trust their counselors to provide them with the information about counseling that they need to make a good decision (Handelsman, 2001). Providing clients with the opportunity to make an informed decision about entering into a counseling relationship demonstrates respect for a client's autonomy and self-determination and is integral to the formation of an effective therapeutic alliance. By communicating the client's role in making treatment decisions, informed consent can increase the client's sense of ownership over the process, establish that the counselor and the client are partners working toward common goals, and reduce the client's anxiety by demystifying the counseling process (Fisher & Oransky, 2008; Margolin et al., 2005). The more that clients understand how counseling works, the more they are likely to benefit from the counseling process (Corey et al., 2024).

The ACA *Code of Ethics* spells out in considerable detail the elements that need to be included in securing informed consent (ACA, 2014, §A.2.b.). At a minimum, the client must be informed about:

- The purposes, goals, techniques, procedures, limitations, potential risks, and benefits of counseling services
- The counselor's qualifications, including relevant degrees held, licenses and certifications obtained, areas of counseling specialization, counseling experience, and approach to counseling
- Arrangements for continuation of services if the counselor dies or becomes incapacitated
- The role of technology
- The implications of diagnosis and the intended use of tests and reports

- Fees and billing information, including procedures for nonpayment of fees
- Confidentiality and its limitations
- Clients' rights to obtain information about their records and to participate in ongoing counseling plans
- Clients' rights to refuse any recommended services or modality change and to be advised of the consequences of refusal

In addition to the elements addressed by the *Code*, we recommend that counselors begin the informed consent process with new clients or prospective clients by offering a brief statement of the counselor's philosophy (how the counselor sees the counseling process taking place). A counselor can offer a simple sentence that begins with "I see counseling as …" to set the tone for a discussion that clarifies shared expectations of the process. For example, clients will be better able to make a choice about entering counseling if they are aware that they will be asked to do homework between sessions, complete assessments, or try out new behaviors outside of the sessions (Wheeler & Bertram, 2019). Clients also need to be informed about fees. They need to understand the implications of insurance reimbursement, including the fact that any assigned diagnosis will become part of their permanent health record. The No Surprises Act, passed in 2022, requires all providers of medical care, including counselors, to inform clients about fee arrangements, anticipated length of treatment, and potential additional fees. Clients are entitled to know about the codes of ethics to which the counselor subscribes and how to obtain a copy of those codes (Wheeler & Bertram, 2019).

The National Board for Certified Counselors (NBCC) has joined with Chi Sigma Iota to provide a useful booklet that describes clients' rights and responsibilities (www.nbcc.org/ethics). This booklet incorporates the elements specified in the ACA *Code of Ethics* (2014), as well as some additional client rights, and presents them in language that is clear and suitable for most clients to understand. The booklet also describes the client's responsibilities in the counseling relationship. Presenting the information in this way helps to establish that counseling is a shared endeavor in which both parties have rights and responsibilities. You may be thinking that a lot of information must be included in the informed consent process, and, indeed, this is true. For clients to be able to give truly informed consent, they must have considerable information before deciding whether to enter a counseling relationship. You may be concerned that inundating prospective clients with information will overwhelm them and give the impression that the counseling relationship is nothing more than a complex, contractual business arrangement. In fact, it is difficult for counselors to achieve the delicate balance between giving clients too little information and giving them so much information that they feel overwhelmed or intimidated (Barnett et al., 2007).

Origins of Informed Consent

As a legal concept, the process of informing recipients of professional services and obtaining their consent to receive such services is relatively new. The requirement that health professionals obtain informed consent from their clients before rendering services began in medicine (Appelbaum et.al., 1987).

Fifty years ago, it probably rarely occurred to physicians that they should explain to patients what the physician knew about the patient's medical conditions, tell patients about various treatment options, or allow patients to decide which option to choose. Perhaps consumers of medical services were once less educated or less aware of their right to receive such information. It has been only in the last half of the twentieth century that courts have created the requirement that patients must be educated or informed regarding their medical treatment options and consequences before they are able to give valid consent to treatment that is legally binding. The first case in the United States that took this position was *Salgo v. Leland Stanford Jr. Univ. Bd. of Trustees* (1957).

The *Salgo* case provided the essential information that physicians must give to patients. According to Appelbaum et al. (1987), these elements are "disclosure of the nature of the ailment, the nature of the proposed treatment, the probability of success, and possible alternative treatments" (p. 41). *Canterbury v. Spence* (1972), a later case, took a different position on informed consent: It found that physicians were required to disclose information about a proposed treatment that a reasonable person, such as the patient receiving treatment, would find necessary to decide to either accept or refuse treatment.

After considering the case law on informed consent, Berner (1998) has argued that there are two elements to the informed consent legal standard: the professional element and materiality. The *professional element* is defined as information that a reasonable physician would provide to another patient in similar circumstances. *Materiality*, on the other hand, is defined as the amount of information that the average patient would consider adequate in deciding whether to accept the treatment. If Berner is correct, then courts will most likely require that physicians provide basic information to all patients *and* require that physicians ensure that the particular patient with whom they are dealing understands the information.

The Health Insurance Portability and Accountability Act (HIPAA), which went into effect April 15, 2003, has had a powerful impact on informed consent practices in medicine and mental health. This federal law and its regulations are explored in detail in Chapter 6. Essentially, HIPAA requires all health care providers who transmit records electronically (which most likely includes all providers) to comply with procedures to ensure patient privacy. The requirements include explaining to health care recipients in detail their rights related to privacy and records (which essentially is the informed consent process discussed earlier). This federal law has thus transformed the concept of informed consent, which once was rather vague, into a concrete framework that includes disclosure of steps taken to ensure client privacy, and it also requires clients to sign a document stating that they have been informed of their rights. Figure 4-1 includes a list of essential elements of informed consent documents to ensure that counselors and agencies meet the HIPAA requirements.

At present, there have been no appellate legal cases involving the responsibility of mental health professionals to obtain informed consent from their clients. But like other legal areas in

FIGURE 4-1 HIPAA requirements for informed consent disclosure statements

To comply with HIPAA, here are some areas you must include on the form that your clients sign stating that they have been informed about your proposed treatment and agreeing to the treatment and the arrangement with you:

- ✓ State that your client's personal information may be used and disclosed to complete treatment. Also state that information may be provided to health care companies related to payment for your services.
- ✓ Develop a complete written description of the procedures you will follow in your office regarding the keeping of or disclosing personal information of clients.
- ✓ Tell your client that you have a more complete description of the way in which you will keep or disclose their personal information and that this complete description is available for them to see. State that the client has a right to review the complete description before signing this consent form. Explain that your practices may change in the future and that if the client wants to see these revisions, the client must make that request in writing, after which you will make the revisions available.
- ✓ Tell your client that they may request that you restrict how their personal information is used or disclosed. Explain that you will consider any such requests and will notify them whether you have agreed to their request.
- ✓ Explain that the client has the right to revoke their consent in writing, except to the extent that actions have already been taken by you based on prior consent.
- ✓ Get the client's signature, and have them indicate the date on the form.
- ✓ Keep the form for at least 6 years.

mental health, most of the precedents and rules are created first with physicians and later become verified for mental health professionals through cases involving psychologists, counselors, and social workers. Nonetheless, it is probably safe to conclude that counselors do have a legal obligation to obtain informed consent from their clients before they begin treatment with them. Further, the informed consent should include information that would be given to the client by other professionals and should be delivered in a way that the client understands. HIPAA also requires that the informed consent of clients be verified with their signatures.

Written Disclosure Statements

In mental health, written contracts for informed consent are commonly referred to as *disclosure statements.* These statements disclose to clients the nature of the counseling relationship into which they are entering. Disclosure statements are also legal contracts that are signed by both the client and the counselor. Many agencies and other settings in which counselors are employed have standard disclosure statements that must be used. For counselors who are self-employed or who must create their own statements, Appendix B includes model disclosure statements for counselors who are providing counseling services to clients in a private practice and for employed counselors who are providing counseling services to clients in an agency or mental health center, respectively. The model counseling disclosure statements for private practice and for agencies or mental health centers are similar in many ways. However, fee issues and some other aspects of the disclosure statements are quite different. Additional disclosure statements found in Appendix B are for counselors who evaluate individuals, who counsel mandated or involuntary clients, and who counsel in schools.

Beyond the informed consent areas required by the ACA *Code of Ethics* (2014) and areas suggested in the literature, some legal concerns need to be addressed in disclosure statements. The following informed consent issues are particularly sensitive and could lead to legal problems if not handled properly. Examples of problems that could arise are included. A client might have the basis for a lawsuit against a counselor if the client believes, for some reason, that the counselor had done any of the following:

1. ***Failed to include required HIPAA elements (see Chapter 6).*** Example: A client was not notified when they began counseling that they had the right to review their counseling records. The client complained to the federal government, and the counselor subsequently was asked to verify that they had informed the client of these rights.
2. ***Guaranteed an outcome as a result of counseling.*** Example: A wife agreed to enter marriage counseling with her husband. She believed that, by agreeing to engage in counseling, she was guaranteed that her marriage would not end. She says the counselor told her that counseling was "the only thing that would save her marriage." She felt betrayed by the counselor when, after five sessions, her husband moved out of their home and filed for divorce. She sues the counselor for breach of contract, conspiracy, and alienation of affection.
3. ***Guaranteed privacy with no exceptions.*** Example: A new client expresses concern to their counselor that others might find out what they tell the counselor in their sessions. To reduce the client's anxiety, the counselor explains the ethical and legal responsibilities that exist to protect the client's privacy. The client subsequently believes that the counselor stated that they would never, under any circumstances, reveal to anyone else information shared in their counseling sessions. Later the counselor informs the client's spouse, over the client's objection, that the client has expressed suicidal ideas and that the counselor believes that the client should be evaluated to determine whether they are at risk. The client sues the counselor for breach of contract, malpractice, and intentional infliction of emotional distress.

4. ***Agreed to fee arrangements different from what was intended.*** Example: A counselor begins a counseling relationship with a client, charging the client the counselor's standard rate of $75 per hour. After 3 months of sessions once a week, the counselor tells the client that the fees will be increased in 1 month to $90 per hour. The client objects, saying that they believe the new hourly rate is too high. The counselor replies that they are not willing to provide further counseling services to the client unless the client agrees to pay the new rate. The client sues the counselor for breach of contract and abandonment.
5. ***Touched without implied or actual permission.*** Example: During a group counseling experience, a counselor asks group members, who are blindfolded, to allow themselves to fall backward into the arms of another group member to demonstrate the difficulty of trusting another person. A female group member reluctantly participates and is caught by a male group member. The female group member is very upset after the exercise and leaves the session visibly shaken. The female client sues the counselor for breach of contract, breach of fiduciary duty, assault, battery, and sexual assault.
6. ***Misrepresented credentials.*** Example: A client begins counseling sessions with a master's-level licensed counselor. The client tells their family members and friends that they are seeing a psychologist for therapy. The client makes out their checks to "Dr.," notes on the checks that they are paying for "psychological services," and gives the checks each week to the office receptionist, who deposits them. At the ninth session, the client calls the counselor "Doctor," and the counselor corrects the client. The client says, "You are a psychologist, aren't you?" The client is very upset when they realize that the counselor is a master's-level licensed counselor. The client sues the counselor for breach of contract and fraudulent misrepresentation.
7. ***Failed to communicate the nature of counseling.*** Example: A client has taken a new job in which they are required periodically to give presentations to small groups of potential clients. The client is very anxious about speaking before groups and has sought counseling to overcome the associated anxiety. The counselor believes that the client's anxiety is the result of low self-esteem and focuses on positive reinforcement of the client's positive attributes. After five sessions, the client complains that they are still as anxious as they were when they began counseling. The client tells the counselor that they had expected to be taught how to give presentations without being anxious and does not understand why the counselor has not provided books to read, given advice on how to be less anxious, or helped them practice making presentations during sessions. The client sues the counselor for breach of contract and malpractice.
8. ***Neglected to warn client about possible stigma.*** Example: After the first session with a new client, the counselor completes an insurance form at the client's request. The counselor indicates on the form that the client has had a single episode of depression and assigns the appropriate *DSM* diagnosis. The counselor counsels the client for 10 sessions, terminating the sessions after the client moves to a new city hundreds of miles away. Two years later the client contacts the counselor and complains that they have been denied a security clearance for a job they have applied for because the counselor diagnosed them, without their knowledge or consent, with a mental disorder. The client sues the counselor for breach of contract, malpractice, and defamation.

These eight hypothetical situations illustrate the importance of ensuring that counselors and their clients have the same understanding regarding their relationship before counseling begins. All of these problems could have been prevented if the counselors had fully informed the clients regarding the counseling relationship. Since HIPAA has been implemented, all counselors must give their clients written informed consent documents to sign. To be effective, however, these documents must include essential information beyond the HIPAA requirements.

4-3 The Case of Ibrahim

Ibrahim, a new licensed professional counselor (LPC), works in a very efficient community mental health agency that utilizes the latest technology. Before he begins his first counseling session with each client, Ibrahim types in the client's name and prints out a personalized disclosure statement that includes all of the HIPAA, ethical, and legal requirements for such documents. Ibrahim is counseling Maureen, a young mother, who discloses that her husband, Jake, gets angry with their infant daughter when she cries and shakes the child to stop her crying. Ibrahim informs Maureen that he must report Jake's actions to the authorities as possible child abuse. Ibrahim, being sensitive to Maureen's concerns, explains in detail what may happen as a result of the report and assures her that he will continue counseling her. Maureen complains that she never would have told Ibrahim about Jake's actions if she had known that Ibrahim would make a report to authorities. Ibrahim responds that Maureen signed the informed consent form when they began their counseling relationship and that the form clearly states that counselors are legally obligated to report incidents of suspected child abuse. Maureen responds that she didn't read the document but just signed it along with all those other papers for insurance. Maureen says she will not continue her counseling relationship with Ibrahim because she no longer trusts him.

- What could Ibrahim have done at the outset of his counseling relationship with Maureen that might have prevented this unfortunate situation? What would have constituted *best practice* in this situation?
- How can counselors balance the need to inform new clients of the limits of confidentiality in the relationship and to establish rapport at the same time?

Discussion: Ibrahim might have considered the following actions to prevent this unfortunate situation:

- Ibrahim might have spent some time with Maureen in their first session pointing out some of the major exceptions to confidentiality. One such major exception is a counselor's legal duty to report suspected child abuse.
- Ibrahim might have asked the intake person in his agency (if such help had been available) to go over some of the major details in the documents with Maureen before he began his first session with her.

To discuss confidentiality limits and establish rapport, counselors should consider taking the following actions:

- Focus on establishing rapport in the first session, and, at the end of the session, go over important details regarding the counseling relationship.
- Highlight in bold type or large type the most important parts of the many papers that clients sign in order to draw clients' attention to those parts.
- Create a written brochure that explains the counseling relationship, including confidentiality limits, and send the brochure to prospective clients for them to read before their first appointment.

In the process of having clients sign counseling disclosure statements, counselors should attempt to ensure that clients understand what is in the documents being signed. If questions arise later about what counselors may have said regarding the counseling sessions, a signed written document establishes the intents of the parties much more clearly than does a statement of what the counselor or the client says they intended. In addition, counseling disclosure statements can be used to correct misunderstandings on the part of clients before counseling begins.

Written disclosure statements allow counselors to provide detailed information. Oral exchanges between counselors and potential clients can then focus on areas that the counselor wishes to emphasize or that cause particular concern for clients.

As noted, every counselor should have a professional disclosure statement that can be given to clients. A complete and signed disclosure statement will fulfill a counselor's legal obligation to obtain informed consent from a client. Keep in mind, though, that disclosure statements are necessary but not sufficient strategies for securing genuine informed consent. They cannot serve as a substitute for a dialogue with the client. Counselors cannot rely on standard forms, no matter how well they are written, to serve as the sole means of securing informed consent. Prospective clients are likely to have many questions about informed consent, and some of these questions are best addressed in face-to-face conversations. Such a dialogue gives clients an opportunity to clarify any information that seems confusing to them and gives counselors the opportunity to gauge the extent to which the clients comprehend the information. At the outset of the relationship, informed consent discussions present a mutual opportunity for the client and the counselor to ensure that they understand their prospective shared journey.

Informed consent procedures must be sensitive to the client's ability to understand. The ACA *Code of Ethics* (ACA, 2014) affirms the obligation of counselors to communicate information in ways that are both developmentally and culturally appropriate (§A.2.c.). To ensure client comprehension, it may be necessary in some cases to arrange for a qualified interpreter or translator. Counselors are cautioned, though, that having a translator present may not be sufficient to ensure the client's informed consent if the translator lacks understanding of the counselor's role and the purpose of the informed consent information. Paone and Malott (2008) have advised counselors to have a pre-session briefing with a translator to review informed consent concepts and how these concepts should be communicated to the client.

Some clients may be presumed to lack the capacity to give informed consent. Such clients might include minors or adults who are developmentally disabled or who suffer from cognitive impairments or a severe thought disorder. In these cases, counselors must seek the consent of legal guardians. Counselors should also seek clients' assent to services and include clients in the decision-making process to the extent possible (ACA, 2014, §A.2.d.). Bennett et al. (2006) have advised counselors to view informed consent capacity as being on a continuum rather than as an all-or-none ability. The informed consent process should be designed to fit each client's cognitive strengths, vulnerabilities, and decision-making capabilities (Fisher et al., 2006).

Managed care arrangements present some specific difficulties in acquiring informed consent. Clients often do not understand how their health plans affect the duration of their treatment, the implications of a diagnosis (Kress et al., 2013), or the amount or nature of the information their insurance providers require counselors to provide. Clients who participate in managed care health insurance plans need to know that the insurer may request their diagnosis, results of any tests utilized, treatment plans, and other clinical information (Corey et al., 2024). Because it would be impossible for counselors to know the details of each of the health care plans of their clients, it is preferable for counselors to stress to clients that they should themselves investigate the terms and limits of their coverage rather than have their counselors undertake that entire burden. Once the

provisions of the plan are known, counselors should discuss with clients the limitations of their coverage; for example, perhaps they will need to agree on treatment goals that are more limited than they would be if cost were no object (Vasquez et al., 2008). See Chapter 13 for further discussions regarding managed care.

From an ethical perspective, ensuring informed consent is not a one-time event. Rather, it is an ongoing process that begins with the initial contact between client and counselor and continues throughout the counseling relationship. It is not always possible for counselors to provide complete informed consent information during the first session. For example, insurance policies often dictate the number of sessions that will be reimbursed, and counselors may not be able to accurately inform clients about the duration of reimbursed counseling until they receive the necessary information from the client's managed care company. Specific goals of counseling and the techniques that may be used to accomplish those goals cannot always be clearly formed during the initial meeting. Some elements of informed consent are best left until a trusting therapeutic relationship has been established; for instance, a client who suffers from a phobia might become anxious when told about exposure therapy, a technique that has proven effective for treating phobias but includes elements (such as getting on an elevator) that might seem frightening at first (Fisher & Oransky, 2008). The ACA *Code of Ethics* (ACA, 2014) describes informed consent as an ongoing part of the counseling process and advises counselors to document discussions of informed consent throughout the counseling relationship (§A.2.a.). As counseling progresses, the goals, issues, risks, and benefits often change, and clients need to have updated information so that they can continue to make sound decisions regarding their treatment (Handelsman, 2001).

It is impossible to predict which clients will need what information and when they will need it, so it is best to be as thorough as possible in a written disclosure statement. Later, in a face-to-face discussion, issues that arise frequently in one's practice can be emphasized, and rare events can be given less attention. For example, Weinrach (1989) has suggested that the two-most-common problems for private practitioners concern (a) fees and billing and (b) late cancellations or no-shows. Thus, it behooves counselors in private practice to be clear with their clients about methods of payment and policies regarding missed appointments.

Research supports the wisdom of securing informed consent. Studies have suggested that clients want information about their prospective counselors (Braaten et al., 1993; Hendrick, 1988); that they perceive counselors who provide informed consent information as being more expert and trustworthy (Sullivan et al., 1993); and that clients who have received appropriate information are more likely to follow their treatment plans, to recover more quickly, to be less anxious, and to be more alert to possible negative consequences (Handler, 1990). In addition, some legal problems can be prevented by the use of disclosure statements. If allegations do arise that a counselor did not fully explain the counseling relationship to a client, a disclosure statement signed by the client often can go a long way toward vindicating an accused counselor who has done no wrong.

DIVERSITY CONSIDERATIONS It is important for us as counselors to question whether the individualism inherent in our concept of informed consent is truly respectful of people of all cultures. We need

to be aware that the full and truthful disclosure that counselors value may be at variance with some clients' cultural beliefs about hope and wellness, that autonomous decision making may run counter to family-centered or group-centered values, and that uncoerced choices may contradict cultural norms about obedience to the wishes of elders or spouses.

4-4 The Case of Henry

Joseph, a Navajo Indian who espouses the traditions of his culture, is Henry's new client. During the initial interview, Henry wants to secure Joseph's informed consent, which would include Henry giving Joseph information about the implications of diagnosis and the potential risks associated with counseling. Henry says, "Sometimes clients in counseling seem to feel worse before they feel better." He adds that based on the symptoms Joseph has described, Joseph seems to be experiencing mild depression.

- What would Henry need to know about Joseph's culture that would help him conduct informed consent procedures in a culturally sensitive way?
- Can you identify any problems with what Henry told Joseph?

Discussion: Henry should have made an effort to gain some basic understanding of the Navajo culture before meeting with Joseph. In traditional Navajo culture, it is believed that language has the power to shape reality and control events. Therefore, Henry, by following his standard informed consent procedures, could be *creating* the reality for Joseph that he is depressed and thus will feel worse before he feels better.

4-5 The Case of Loretta

Soo Jung is a 22-year-old immigrant from Korea who is brought to the counseling center by her husband. During the intake interview, she tells her counselor, Loretta, that she has been having crying spells, has lost her appetite, and is not sleeping well. She gave birth to her first child 2 months earlier. When Loretta suggests to Soo Jung that she may be experiencing postpartum depression, Soo Jung becomes upset and says, "Why are you telling me this? You need to talk to my husband."

- Why do you think Soo Jung became upset?
- How might Loretta have handled the intake interview differently to avoid upsetting this client?

Discussion: Soo Jung appears to share a family-oriented world view that is common among people from some Asian cultures. Because the counselor did not include this client's husband in this discussion, Soo Jung may have interpreted the counselor's actions as attempting to undermine her marital relationship. An awareness of the Korean culture and its views on marriage might have led Loretta to interview Soo Jung and her husband together as the counseling relationship was being established.

There are communities of clients in our pluralistic society who do not claim Euro-American values as their own. Counselors must also remember to put the client's needs first. Lasser and Gottlieb (2017) remind counselors that they must take into consideration the frame of reference or belief and value systems that are central to the client to remain sensitive to cultural differences. Expanding our frame of reference regarding informed consent does not mean that we must abandon our commitment to client autonomy, but it does encompass respect for the cultural values that our clients bring with them to counseling.

Counselors must also be sensitive to clients' rights to informed consent when clients have been legally adjudicated as incompetent. Clients with developmental disabilities, older adults who have cognitive impairments, and persons who have been diagnosed with a psychotic or thought disorder often are judged to have diminished capacity to consent. Keep in mind that *diminished* capacity is not the same as *an absence of* capacity. Counselors have an ethical obligation to discuss consent with these clients in a manner that they can understand and to obtain their assent to services even though their agreement may not be legally recognized (Fisher & Oransky, 2008; Handelsman, 2001).

The importance of using language that clients can understand is equally important with clients who are children, adults who are not well educated, and clients whose first language is not English. If you work with such clients, it would be wise to carefully check the readability level of your written disclosure statement. When English is a second language for clients, it would be helpful to have a copy that has been translated into the client's primary language. The overarching principle is that consent procedures, both oral and written, must be developmentally and culturally appropriate for the client.

Summary and Key Points

This chapter addresses two vitally important and interrelated ethical and legal issues in counseling: client welfare and informed consent.

Following are key points regarding client welfare:

- Counselors are ethically responsible for putting client welfare first and foremost in their work.
- Counselors have a fiduciary relationship with their clients, which means that they have a legal obligation to protect their clients' best interests and to prevent interactions that benefit themselves.
- Respecting diversity is fundamental to protecting client welfare and promoting client dignity in our pluralistic society.
- It is vital for counselors to be aware of their own needs and motivations, unfinished business, areas of vulnerability, and defenses so that they will not inadvertently be working on their own issues during the counseling sessions rather than on the client's issues.
- Counselors must be aware of their personal values and must not impose their values on clients. Recent court cases have highlighted issues related to value conflicts between counselors and clients.
- Counselors must avoid fostering client dependency. They have an obligation instead to promote client autonomy and independence.
- When counseling involuntary or mandated clients, counselors must inform these clients of the limits of confidentiality and the nature of any reports that may be made to courts or other entities that are requiring the clients to attend counseling. Counselors must be sensitive to these clients' rights and ensure that these clients understand any privacy rights they may be waiving by taking part in counseling.
- Counselors have an obligation to select appropriate counseling strategies or techniques and to make clients active partners in treatment decisions.

- Counselors must protect the best interests and welfare of clients when counseling services are interrupted or terminated prematurely.
- It is unethical, as well as illegal, for counselors to abandon their clients. Although there are valid reasons for a counselor to end a counseling relationship, care must be taken not to put the client at risk when doing so. Several guidelines were offered for counselors to use when they are terminating counseling relationships in adverse circumstances.

The following are key points regarding informed consent in counseling:

- As an ethical obligation, informed consent is based on the rationale that clients have the right to know what they are undertaking when they come for counseling.
- Counselors should understand that counseling services are contractual in nature. Agreements should thus be put in writing in an informed consent document.
- The requirement that health professionals inform prospective recipients of services and obtain their consent to treatment originated in the field of medicine. More recently, it has been applied to the mental health field. A number of model disclosure statements are available to assist counselors who work in various settings and with a variety of clientele.
- The requirements of HIPAA have transformed informed consent into a legal requirement, and counselors must ensure that their procedures are in compliance with this law.
- Informed consent in counseling involves a number of elements that are required by the ACA *Code of Ethics*, recommended by various researchers, and suggested as a means to address legal concerns.
- Counselors need to be aware of at least eight areas of informed consent that, if not handled properly, could lead to legal problems.
- It can be difficult to achieve a balance between providing prospective clients with too little information and giving them so much information that they feel overwhelmed or intimidated. Informed consent is an ongoing process that should be conducted both orally and in written form.
- It is important for counselors to be sensitive to the fact that informed consent as traditionally conceptualized may not be respectful of people of all cultures and to educate themselves as appropriate for their clients.